



Daily Note

Date of Visit: 02-26-26
Patient Name: Hocke-Mirzashvili, Tatjana
Patient #: TX0458176

Therapist: Christobal Saenz
Referring MD: Martha Pyron
 Phone: (512) 676-2500
 Fax: (512) 406-7377

Date of Birth: 12-15-1982
Age: 43
Certification Period: 02-11-26 - 04-07-26

Time In:
Time Out:

Date of Onset: 09-01-25

Diagnosis:

M79.671 Pain in right foot
 M79.672 Pain in left foot
 M25.562 Pain in left knee
 M22.2X2 Patellofemoral disorders, left knee
 M54.50 Low back pain, unspecified

SUBJECTIVE:

Comments: 2/10 R foot pain currently

2/26/26: Pt reports that her right ankle pain has been worse than usual. Overall however, her heel and back pain have been doing better, not quite normal but getting close.

2/24: Patient states that her pain hovers around 1/10 and sometimes its also gone which feels very weird as she is so used to the pain. She is doing her HEP everyday.

2/19/26: Pt reports that her feet are feeling better. She has less issues at rest, but standing still bothers her. She does have a headache, but feels that that is not related.

2/17: Patient went to Zoo over the weekend and her feet hurt but she is doing the exercises. She feels good with exercises. R heel pain 2/10

Patient saw MD last week after talking to her therapists. X ray was taken which showed she has arthritis and disc herniation but PCP doesn't think patient needs new imaging as they don't anticipate big changes in it. Patient was given referral for her lumbar spine. Patient reports being "good for a while" initially but experienced a flare-up after increasing exercise activities over the summer, including swimming and yoga. Patient notes difficulty isolating pelvic floor muscles during breathing exercises, with tendency to activate upper abdominal muscles instead. Sleep quality is affected, requiring soft padding for comfort. Patient demonstrates understanding of exercise modifications and reports compliance with wall stretches when feeling stiff.

Contraindications:

- None

ACTIVITY LOG:

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Time Based Activities	Comment	Status	Sets	Reps	Weight	Time	Done Today	Special	CPT
Calf stretch on step						6	Yes		Neuro Re-Ed (97112) - [B02]
One leg bridge						4	Yes		Therapeutic Activity (97530) - [B12]
Myofascial Release	Calf/ soleus medial and lateral	Active				20	Yes		Neuro Re-Ed (97112) - [B02]
Patient education	ergonomics for cycling	Active				5	Yes		Therapeutic Activity (97530) - [B12]
dead bugs		Active				5	Yes		Neuro Re-Ed (97112) - [B02]

ASSESSMENT:

Patient consents to treatment plan and goals and gives verbal informed consent. 2/26/26: Pt's right ankle pain is reproduced with palpation of the soleus and peroneals and responds well to myofascial release. She has good forms with dead bugs but rapid fatigue. Her snapping feeling reduces with cues for increased knee flexion during exercise.

2/14: Plantar fascia and peroneal release with gua sha performed as patient c/o soreness and tightness. Taping for heel stabilization done today with care instructions given to patient. Dead bugs were added today and patient was appropriately challenged with them. She c/o snapping feeling in her R hip flexor area so hip flexor stretch was added as well.

2/19/26: Pt with improvements in her soft tissue irritability and core strength today.

2/17/26: Manual lumbar traction performed and patient tolerated it well. MFR performed on glutes/piriformis to release tightness.

2/12/26: Pt with improvements in her core strength and motor control.

2/5/26: Pt with reproduction of her pain with palpation of the Achilles tendon. With palpation she has signs and symptoms consistent with diastasis recti with pain reproduction with abdominal distention.

2/2/2026

The tx session began with cobra pose, followed by soft tissue mobilization the the thoracic and lumbar spine. Pt reported increased tenderness more distally along the lumbar spine. Childs pose and open books were performed to facilitate tissue extensibility of the thoracic and lumbar spine. Pt reported feeling decreased symptoms in bilateral feet after the tx session. Pt education provided on discussing low back findings with physician and whether or not imaging would be recommended. Pt's last MRI of the spine was performed 2 years ago.

1/29/26: Pt's back and feet symptoms continue to respond to McKenzie extension interventions. Her knee pain does improve with tibial IR mobilization, indicating mechanical left knee pain.

1/21/26: Today, the patient's foot pain, numbness, and weakness improves with lumbar PA glides as well as with myofascial release of the piriformis. These signs and symptoms are consistent with neurological origin of symptoms. At next visit, plan to continue to address lumbar radicular symptoms and work on improving core and hip strength.

Pt is a 42 year old female who reports gradual onset of B heel pain and L knee pain that has been ongoing since 9/2025. Pt demonstrates decreased B heel pain that is indicative of a relevant lumbar component as demonstrated by decreased B heel pain from a 6/10 to a 2/10 with pain moving from heel to just above calcaneus. Pt continues to report occasional numbness and tingling in feet with prolonged sitting.

Goals	Short-Long	Time Frame	Result	Comment
Patient will be independent in progressive monitored HEP.	Short Term	1 week	Met	
Improve position tolerance of walking on uneven terrain	Long Term	8 weeks	Ongoing	
Improve L single leg stance time to 30 seconds	Long Term	8 weeks	Ongoing	
Pt will stand 30 minutes pain free	Long Term	8 weeks	Partially Met 1-24%	
Patient will be independent with HEP	Short Term	4 weeks		

Patient: Tatjana Hocke-Mirzashvili

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Goals	Short-Long	Time Frame	Result	Comment
Patient will demonstrate increase in L/S AROM in all directions w/o pain	Long Term	8 weeks		
Centralization of radicular symptoms	Long Term	8 weeks		
Eliminate post innominate rotation on R Side	Short Term	4 weeks		

PLAN:

Tatjana will continue therapy as prescribed. This is visit # 25.

Best regards,

Christobal Saenz PT

This document was electronically signed on 02-26-26 at 09:50a by Christobal Saenz PT Lic# TX48758PT



Daily Note

Date of Visit: 02-24-26
Patient Name: Hocke-Mirzashvili, Tatjana
Patient #: TX0458176

Therapist: Namrah Quazi
Referring MD: Martha Pyron
 Phone: (512) 676-2500
 Fax: (512) 406-7377

Date of Birth: 12-15-1982
Age: 43
Certification Period: 02-11-26 - 04-07-26

Time In:
Time Out:

Date of Onset: 09-01-25

Diagnosis:

M79.671 Pain in right foot
 M79.672 Pain in left foot
 M25.562 Pain in left knee
 M22.2X2 Patellofemoral disorders, left knee
 M54.50 Low back pain, unspecified

SUBJECTIVE:

Comments: 2/10 R foot pain currently

2/24: Patient states that her pain hovers around 1/10 and sometimes its also gone which feels very weird as she is so used to the pain. She is doing her HEP everyday.

2/19/26: Pt reports that her feet are feeling better. She has less issues at rest, but standing still bothers her. She does have a headache, but feels that that is not related.

2/17: Patient went to Zoo over the weekend and her feet hurt but she is doing the exercises. She feels good with exercises. R heel pain 2/10

Patient saw MD last week after talking to her therapists. X ray was taken which showed she has arthritis and disc herniation but PCP doesn't think patient needs new imaging as they don't anticipate big changes in it. Patient was given referral for her lumbar spine. Patient reports being "good for a while" initially but experienced a flare-up after increasing exercise activities over the summer, including swimming and yoga. Patient notes difficulty isolating pelvic floor muscles during breathing exercises, with tendency to activate upper abdominal muscles instead. Sleep quality is affected, requiring soft padding for comfort. Patient demonstrates understanding of exercise modifications and reports compliance with wall stretches when feeling stiff.

Contraindications:

- None

ACTIVITY LOG:

Time Based Activities	Comment	Status	Sets	Reps	Weight	Time	Done Today	Special	CPT
Myofascial Release	piriformis bil	Active				4	Yes		Neuro Re-Ed (97112) - [B02]

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Time Based Activities	Comment	Status	Sets	Reps	Weight	Time	Done Today	Special	CPT
hip flexor stretch	2x30"	Active				4	Yes		Neuro Re-Ed (97112) - [B02]
Joint Mobilization	PA lumbar glides/ SI joint AP and inferior glide R side	Active				5	Yes		Neuro Re-Ed (97112) - [B02]
Patient education		Active				6	Yes		Therapeutic Activity (97530) - [B12]
Taping	Heel support	Active				6	Yes		Neuro Re-Ed (97112) - [B02]
dead bugs		Active				5	Yes		Neuro Re-Ed (97112) - [B02]
Soft Tissue Mobilization	R heel and plantar fascia with gua sha release	Active				10	Yes		Neuro Re-Ed (97112) - [B02]

ASSESSMENT:

Patient consents to treatment plan and goals and gives verbal informed consent. 2/14: Plantarfascia and peroneal release with gua sha performed as patient c/o soreness and tightness. Taping for heel stabilization done today with care instructions given to patient. Dead bugs were added today and patient was appropriately challenged with them . She c.o snapping feeling in her R hip flexor area so hip flexor stretch was added as well.

2/19/26: Pt with improvements in her soft tissue irritability and core strength today.

2/17/26: Manual lumbar traction performed and patient tolerated it well. MFR performed on glutes/piriformis to release tightness.

2/12/26: Pt with improvements in her core strength and motor control.

2/5/26: Pt with reproduction of her pain with palpation of the Achilles tendon. With palpation she has signs and symptoms consistent with diastasis recti with pain reproduction with abdominal distention.

2/2/2026

The tx session began with cobra pose, followed by soft tissue mobilization the the thoracic and lumbar spine. Pt reported increased tenderness more distally along the lumbar spine. Childs pose and open books were performed to facilitate tissue extensibility of the thoracic and lumbar spine. Pt reported feeling decreased symptoms in bilateral feet after the tx session. Pt education provided on discussing low back findings with physician and whether or not imaging would be recommended. Pt's last MRI of the spine was performed 2 years ago.

1/29/26: Pt's back and feet symptoms continue to respond to McKenzie extension interventions. Her knee pain does improve with tibial IR mobilization, indicating mechanical left knee pain.

1/21/26: Today, the patient's foot pain, numbness, and weakness improves with lumbar PA glides as well as with myofascial release of the piriformis. These signs and symptoms are consistent with neurological origin of symptoms. At next visit, plan to continue to address lumbar radicular symptoms and work on improving core and hip strength.

Pt is a 42 year old female who reports gradual onset of B heel pain and L knee pain that has been ongoing since 9/2025. Pt demonstrates decreased B heel pain that is indicative of a relevant lumbar component as demonstrated by decreased B heel pain from a 6/10 to a 2/10 with pain moving from heel to just above calcaneus. Pt continues to report occasional numbness and tingling in feet with prolonged sitting.

Goals	Short-Long	Time Frame	Result	Comment
Patient will be independent in progressive monitored HEP.	Short Term	1 week	Met	
Improve position tolerance of walking on uneven terrain	Long Term	8 weeks	Ongoing	
Improve L single leg stance time to 30 seconds	Long Term	8 weeks	Ongoing	
Pt will stand 30 minutes pain free	Long Term	8 weeks	Partially Met	

Patient: Tatjana Hocke-Mirzashvili

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Goals	Short-Long	Time Frame	Result	Comment
			1-24%	
Patient will be independent with HEP	Short Term	4 weeks		
Patient will demonstrate increase in L/S AROM in all directions w/o pain	Long Term	8 weeks		
Centralization of radicular symptoms	Long Term	8 weeks		
Eliminate post innominate rotation on R Side	Short Term	4 weeks		

PLAN:

Tatjana will continue therapy as prescribed. This is visit # 24.

Best regards,

Namrah Quazi PT1360147

This document was electronically signed on 02-24-26 at 09:53a by Namrah Quazi PT1360147 Lic# 1360147



Daily Note

Date of Visit: 02-19-26
Patient Name: Hocke-Mirzashvili, Tatjana
Patient #: TX0458176

Therapist: Christobal Saenz
Referring MD: Martha Pyron
 Phone: (512) 676-2500
 Fax: (512) 406-7377

Date of Birth: 12-15-1982
Age: 43
Certification Period: 02-11-26 - 04-07-26

Time In:
Time Out:

Date of Onset: 09-01-25

Diagnosis:

M79.671 Pain in right foot
 M79.672 Pain in left foot
 M25.562 Pain in left knee
 M22.2X2 Patellofemoral disorders, left knee
 M54.50 Low back pain, unspecified

SUBJECTIVE:

Comments: 2/10 R foot pain currently

2/19/26: Pt reports that her feet are feeling better. She has less issues at rest, but standing still bothers her. She does have a headache, but feels that that is not related.

2/17: Patient went to Zoo over the weekend and her feet hurt but she is doing the exercises. She feels good with exercises. R heel pain 2/10

Patient saw MD last week after talking to her therapists. X ray was taken which showed she has arthritis and disc herniation but PCP doesn't think patient needs new imaging as they don't anticipate big changes in it. Patient was given referral for her lumbar spine. Patient reports being "good for a while" initially but experienced a flare-up after increasing exercise activities over the summer, including swimming and yoga. Patient notes difficulty isolating pelvic floor muscles during breathing exercises, with tendency to activate upper abdominal muscles instead. Sleep quality is affected, requiring soft padding for comfort. Patient demonstrates understanding of exercise modifications and reports compliance with wall stretches when feeling stiff.

Contraindications:

- None

ACTIVITY LOG:

Time Based Activities	Comment	Status	Sets	Reps	Weight	Time	Done Today	Special	CPT
Paloff Press						6	Yes		Therapeutic Activity (97530) - [B12]

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Time Based Activities	Comment	Status	Sets	Reps	Weight	Time	Done Today	Special	CPT
Bridge	3#	Active				4	Yes		Therapeutic Activity (97530) - [B12]
Braced Reverse Marching						6	Yes		Therapeutic Activity (97530) - [B12]
Repeated extension in lying	McKenzie extension level 4 (press up on hands) with belt	Active				4	Yes		Neuro Re-Ed (97112) - [B02]
Posterior Pelvic Tilt		Active				4	Yes		Neuro Re-Ed (97112) - [B02]
Joint Mobilization	PA lumbar glides/ SI joint AP and inferior glide R side	Active				12	Yes		Neuro Re-Ed (97112) - [B02]
Patient education		Active				4	Yes		Therapeutic Activity (97530) - [B12]

ASSESSMENT:

Patient consents to treatment plan and goals and gives verbal informed consent. 2/19/26: Pt with improvements in her soft tissue irritability and core strength today.

2/17/26: Manual lumbar traction performed and patient tolerated it well. MFR performed on glutes/piriformis to release tightness.

2/12/26: Pt with improvements in her core strength and motor control.

2/5/26: Pt with reproduction of her pain with palpation of the Achilles tendon. With palpation she has signs and symptoms consistent with diastasis recti with pain reproduction with abdominal distention.

2/2/2026

The tx session began with cobra pose, followed by soft tissue mobilization the the thoracic and lumbar spine. Pt reported increased tenderness more distally along the lumbar spine. Childs pose and open books were performed to facilitate tissue extensibility of the thoracic and lumbar spine. Pt reported feeling decreased symptoms in bilateral feet after the tx session. Pt education provided on discussing low back findings with physician and whether or not imaging would be recommended. Pt's last MRI of the spine was performed 2 years ago.

1/29/26: Pt's back and feet symptoms continue to respond to McKenzie extension interventions. Her knee pain does improve with tibial IR mobilization, indicating mechanical left knee pain.

1/21/26: Today, the patient's foot pain, numbness, and weakness improves with lumbar PA glides as well as with myofascial release of the piriformis. These signs and symptoms are consistent with neurological origin of symptoms. At next visit, plan to continue to address lumbar radicular symptoms and work on improving core and hip strength.

Pt is a 42 year old female who reports gradual onset of B heel pain and L knee pain that has been ongoing since 9/2025. Pt demonstrates decreased B heel pain that is indicative of a relevant lumbar component as demonstrated by decreased B heel pain from a 6/10 to a 2/10 with pain moving from heel to just above calcaneus. Pt continues to report occasional numbness and tingling in feet with prolonged sitting.

Goals	Short-Long	Time Frame	Result	Comment
Patient will be independent in progressive monitored HEP.	Short Term	1 week	Met	
Improve position tolerance of walking on uneven terrain	Long Term	8 weeks	Ongoing	
Improve L single leg stance time to 30 seconds	Long Term	8 weeks	Ongoing	
Pt will stand 30 minutes pain free	Long Term	8 weeks	Partially Met	

Patient: Tatjana Hocke-Mirzashvili

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Goals	Short-Long	Time Frame	Result	Comment
			1-24%	
Patient will be independent with HEP	Short Term	4 weeks		
Patient will demonstrate increase in L/S AROM in all directions w/o pain	Long Term	8 weeks		
Centralization of radicular symptoms	Long Term	8 weeks		
Eliminate post innominate rotation on R Side	Short Term	4 weeks		

PLAN:

Tatjana will continue therapy as prescribed. This is visit # 23.

Best regards,

Christobal Saenz PT

This document was electronically signed on 02-19-26 at 10:35a by Christobal Saenz PT Lic# TX48758PT



Daily Note

Date of Visit: 02-17-26
Patient Name: Hocke-Mirzashvili, Tatjana
Patient #: TX0458176

Therapist: Namrah Quazi
Referring MD: Martha Pyron
 Phone: (512) 676-2500
 Fax: (512) 406-7377

Date of Birth: 12-15-1982
Age: 43
Certification Period: 02-11-26 - 04-07-26

Time In:
Time Out:

Date of Onset: 09-01-25

Diagnosis:

M79.671 Pain in right foot
 M79.672 Pain in left foot
 M25.562 Pain in left knee
 M22.2X2 Patellofemoral disorders, left knee
 M54.50 Low back pain, unspecified

SUBJECTIVE:

Comments: 2/10 R foot pain currently

2/17: Patient went to Zoo over the weekend and her feet hurt but she is doing the exercises. She feels good with exercises. R heel pain 2/10

Patient saw MD last week after talking to her therapists. X ray was taken which showed she has arthritis and disc herniation but PCP doesn't think patient needs new imaging as they don't anticipate big changes in it. Patient was given referral for her lumbar spine. Patient reports being "good for a while" initially but experienced a flare-up after increasing exercise activities over the summer, including swimming and yoga. Patient notes difficulty isolating pelvic floor muscles during breathing exercises, with tendency to activate upper abdominal muscles instead. Sleep quality is affected, requiring soft padding for comfort. Patient demonstrates understanding of exercise modifications and reports compliance with wall stretches when feeling stiff.

Contraindications:

- None

ACTIVITY LOG:

Time Based Activities	Comment	Status	Sets	Reps	Weight	Time	Done Today	Special	CPT
Lumbar and SI glides	R PA and inferior SI glides; PA glides L/S globally	Active				8	Yes		Neuro Re-Ed (97112) - [B02]
Myofascial	piriformis bil	Active				10	Yes		Neuro Re-Ed

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Time Based Activities	Comment	Status	Sets	Reps	Weight	Time	Done Today	Special	CPT
Release									(97112) - [B02]
Repeated extension in lying	McKenzie extension level 4 (press up on hands) with belt	Active				4	Yes		Neuro Re-Ed (97112) - [B02]
Posterior Pelvic Tilt		Active				4	Yes		Neuro Re-Ed (97112) - [B02]
Manual lumbar tracti		Active				6	Yes		Neuro Re-Ed (97112) - [B02]
Joint Mobilization	PA lumbar glides/ SI joint AP and inferior glide R side	Active				12	Yes		Neuro Re-Ed (97112) - [B02]

ASSESSMENT:

Patient consents to treatment plan and goals and gives verbal informed consent. 2/17/26: Manual lumbar traction performed and patient tolerated it well. MFR performed on glutes/piriformis to release tightness.

2/12/26: Pt with improvements in her core strength and motor control.

2/5/26: Pt with reproduction of her pain with palpation of the Achilles tendon. With palpation she has signs and symptoms consistent with diastasis recti with pain reproduction with abdominal distention.

2/2/2026

The tx session began with cobra pose, followed by soft tissue mobilization the the thoracic and lumbar spine. Pt reported increased tenderness more distally along the lumbar spine. Childs pose and open books were performed to facilitate tissue extensibility of the thoracic and lumbar spine. Pt reported feeling decreased symptoms in bilateral feet after the tx session. Pt education provided on discussing low back findings with physician and whether or not imaging would be recommended. Pt's last MRI of the spine was performed 2 years ago.

1/29/26: Pt's back and feet symptoms continue to respond to McKenzie extension interventions. Her knee pain does improve with tibial IR mobilization, indicating mechanical left knee pain.

1/21/26: Today, the patient's foot pain, numbness, and weakness improves with lumbar PA glides as well as with myofascial release of the piriformis. These signs and symptoms are consistent with neurological origin of symptoms. At next visit, plan to continue to address lumbar radicular symptoms and work on improving core and hip strength.

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Goals	Short-Long	Time Frame	Result	Comment
Patient will be independent in progressive monitored HEP.	Short Term	1 week	Met	
Improve position tolerance of walking on uneven terrain	Long Term	8 weeks	Ongoing	
Improve L single leg stance time to 30 seconds	Long Term	8 weeks	Ongoing	
Pt will stand 30 minutes pain free	Long Term	8 weeks	Partially Met 1-24%	
Patient will be independent with HEP	Short Term	4 weeks		
Patient will demonstrate increase in L/S AROM in all directions w/o pain	Long Term	8 weeks		
Centralization of radicular symptoms	Long Term	8 weeks		
Eliminate post innominate rotation on R Side	Short Term	4 weeks		

Patient: Tatjana Hocke-Mirzashvili

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Phone: (737) 843-7014 Fax: (737) 843-7016

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PLAN:

Tatjana will continue therapy as prescribed. This is visit # 22.

Best regards,

Namrah Quazi PT1360147

This document was electronically signed on 02-17-26 at 11:19a by Namrah Quazi PT1360147 Lic# 1360147



Endeavor Physical Therapy
A H2 HEALTH Company

Daily Note

Date of Visit: 02-12-26
Patient Name: Hocke-Mirzashvili, Tatjana
Patient #: TX0458176

Therapist: Christobal Saenz
Referring MD: Martha Pyron
Phone: (512) 676-2500
Fax: (512) 406-7377

Date of Birth: 12-15-1982
Age: 43
Certification Period: 02-11-26 - 04-07-26

Time In:
Time Out:

Date of Onset: 09-01-25

Diagnosis:

M79.671 Pain in right foot
M79.672 Pain in left foot
M25.562 Pain in left knee
M22.2X2 Patellofemoral disorders, left knee
M54.50 Low back pain, unspecified

SUBJECTIVE:

Comments: 2/10 R foot pain currently

2/12/26: Pt reports that she is feeling overall good. She has a new prescription for her back. Her physician diagnosed her with a disc issue and arthritis. Her foot pain is around a 2/10 but it was about a 4/10 when she had to do a long drive.

Patient saw MD last week after talking to her therapists. X ray was taken which showed she has arthritis and disc herniation but PCP doesn't think patient needs new imaging as they don't anticipate big changes in it. Patient was given referral for her lumbar spine. Patient reports being "good for a while" initially but experienced a flare-up after increasing exercise activities over the summer, including swimming and yoga. Patient notes difficulty isolating pelvic floor muscles during breathing exercises, with tendency to activate upper abdominal muscles instead. Sleep quality is affected, requiring soft padding for comfort. Patient demonstrates understanding of exercise modifications and reports compliance with wall stretches when feeling stiff.

Contraindications:

- None

ACTIVITY LOG:

Time Based Activities	Comment	Status	Sets	Reps	Weight	Time	Done Today	Special	CPT
Isometric Crunch		Active				6	Yes		Therapeutic Activity (97530) - [B12]
Braced Reverse Marching						6	Yes		Therapeutic Activity (97530) -

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Time Based Activities	Comment	Status	Sets	Reps	Weight	Time	Done Today	Special	CPT
									[B12]
Repeated extension in lying	McKenzie extension level 4 (press up on hands) with belt	Active				8	Yes		Neuro Re-Ed (97112) - [B02]
Bridge		Active				4	Yes		Therapeutic Activity (97530) - [B12]
Posterior Pelvic Tilt		Active				4	Yes		Neuro Re-Ed (97112) - [B02]
Joint Mobilization	PA lumbar glides/ SI joint AP and inferior glide R side	Active				12	Yes		Neuro Re-Ed (97112) - [B02]

ASSESSMENT:

Patient consents to treatment plan and goals and gives verbal informed consent. 2/12/26: Pt with improvements in her core strength and motor control.

2/5/26: Pt with reproduction of her pain with palpation of the Achilles tendon. With palpation she has signs and symptoms consistent with diastasis recti with pain reproduction with abdominal distention.

2/2/2026

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Goals	Short-Long	Time Frame	Result	Comment
Patient will be independent in progressive monitored HEP.	Short Term	1 week	Met	
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Improve L single leg stance time to 30 seconds	Long Term	8 weeks	Ongoing	
Pt will stand 30 minutes pain free	Long Term	8 weeks	Partially Met 1-24%	
Patient will be independent with HEP	Short Term	4 weeks		
Patient will demonstrate increase in L/S AROM in all directions w/o pain	Long Term	8 weeks		
Centralization of radicular symptoms	Long Term	8 weeks		
Eliminate post innominate rotation on R Side	Short Term	4 weeks		

Patient: Tatjana Hocke-Mirzashvili

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Phone: (737) 843-7014 Fax: (737) 843-7016

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PLAN:

Tatjana will continue therapy as prescribed. This is visit # 21.

Best regards,

Christobal Saenz PT

This document was electronically signed on 02-12-26 at 12:46p by Christobal Saenz PT Lic# TX48758PT



Progress Note

Date of Visit: 02-10-26
Patient Name: Hocke-Mirzashvili, Tatjana
Patient #: TX0458176

Therapist: Namrah Quazi
Referring MD: Martha Pyron
 Phone: (512) 676-2500
 Fax: (512) 406-7377

Date of Birth: 12-15-1982
Age: 43
Certification Period: 02-11-26 - 04-07-26

Time In:
Time Out:

Date of Onset: 09-01-25

Diagnosis:

M79.671 Pain in right foot
 M79.672 Pain in left foot
 M25.562 Pain in left knee
 M22.2X2 Patellofemoral disorders, left knee
 M54.50 Low back pain, unspecified

SUBJECTIVE:

Presenting Problems:

The patient reports:

- Difficulty ascending/descending stairs, Moderate
- Difficulty walking, Minimal
- Difficulty squatting, Moderate
- Back stiffness, Moderate
- Difficulty isolating pelvic floor muscles, Moderate

Comments: 2/10 R foot pain currently

Functional Status	Prior	Current
Activities of daily living	100%	50%

Contraindications:

- None

Patient saw MD last week after talking to her therapists. X ray was taken which showed she has arthritis and disc herniation but PCP doesn't think patient needs new imaging as they don't anticipate big changes in it. Patient was given referral for her lumbar spine. Patient reports being "good for a while" initially but experienced a flare-up after increasing exercise activities over the summer, including swimming and yoga. Patient notes difficulty isolating pelvic floor muscles during breathing exercises, with tendency to activate upper abdominal muscles instead. Sleep quality is affected, requiring soft padding for comfort. Patient demonstrates understanding of exercise modifications and reports compliance with wall stretches when feeling stiff.

OBJECTIVE:
LUMBAR EVALUATION

RANGE OF MOTION:

2/10 PN:

- Spinal flexion: Patient reports whole body pain/tightness during forward bending with toes up
- Side bending: Right side bending causes discomfort, left side bending tolerated better
- Spinal rotation: Left rotation causes some discomfort in opposite side, right rotation well tolerated

MANUAL MUSCLE TEST:

Lower Extremity	Initial
MMT	02-10-26
Right Hip Abduction	3
Left Hip Abduction	3

PALPATION:

SI joint: R AGMR

R post innominate rotation

TTP: R SI joint.

TTP: Bil lumbar paraspinals

FUNCTION/OBSERVATION:

c/o pain and sensitivity in bil heels

SPECIAL TESTS:

- Straight leg raise test: Positive on left side with pain reported in left side/back area
- Left leg straight leg raise with neck flexion: Increased pain reported in neck area
- Slump test: BIL negative (muscle stretch)

JOINT MOBILITY:

L/S PA glides: Hypomobile and painful lower L/S and L5-S1

ACTIVITY LOG:

Time Based Activities	Comment	Status	Sets	Reps	Weight	Time	Done Today	Special	CPT
Reassessment	Lumbar assessment	Active				15	Yes		Therapeutic Activity (97530) - [B12]
Shotgun SI glide	using dowel	Active				3	Yes		Neuro Re-Ed (97112) - [B02]
Myofascial Release	piriformis	Active				5	Yes		Neuro Re-Ed (97112) - [B02]
Joint Mobilization	PA lumbar glides/ SI joint AP and inferior glide R side	Active				6	Yes		Neuro Re-Ed (97112) - [B02]

Patient: Tatjana Hocke-Mirzashvili

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Time Based Activities	Comment	Status	Sets	Reps	Weight	Time	Done Today	Special	CPT
Patient Education	clinical findings/POC/prognosis/all patient questions were answered	Active				12	Yes		Therapeutic Activity (97530) - [B12]
Pelvic Floor Exercises		Active				2	Yes		Neuro Re-Ed (97112) - [B02]

ASSESSMENT:

Patient demonstrates varied progress across multiple conditions being treated concurrently. For pelvic floor dysfunction, patient shows good understanding of breathing techniques and muscle isolation exercises, though continues to struggle with proper muscle activation and compensatory patterns. Patient reports difficulty separating pelvic floor contractions from upper abdominal muscle activation, indicating need for continued neuromuscular re-education. Patient demonstrates good compliance with home exercise program and motivation for recovery. Patient would benefit from continued skilled therapy to address neuromuscular re-education needs, manual therapy for tissue mobility, and progressive strengthening while managing multiple concurrent conditions. The patient's rehab potential is good. The patient's discharge prognosis is good.

The patient appears motivated and is able to perform exercises correctly with verbal cues only. The patient's progress towards goals is good and her tolerance to treatment is good. Patient consents to treatment plan and goals and gives verbal informed consent.

Goals	Short-Long	Time Frame	Result	Comment
Patient will be independent in progressive monitored HEP.	Short Term	1 week	Met	
Improve position tolerance of walking on uneven terrain	Long Term	8 weeks	Ongoing	
Improve L single leg stance time to 30 seconds	Long Term	8 weeks	Ongoing	
Pt will stand 30 minutes pain free	Long Term	8 weeks	Partially Met 1-24%	
Patient will be independent with HEP	Short Term	4 weeks		
Patient will demonstrate increase in L/S AROM in all directions w/o pain	Long Term	8 weeks		
Centralization of radicular symptoms	Long Term	8 weeks		
Eliminate post innominate rotation on R Side	Short Term	4 weeks		

PLAN:

The patient's treatment will include joint and soft tissue mobilization, neuromuscular re-education, therapeutic activities, Dry Needling, therapeutic exercises, therapeutic activities, gait training and e-Stim unattended no wound. The patient will continue therapy 2 times per week for 8 weeks, for an additional 16 visits and a total of 36 visits since start of care. This is visit # 20.

Thank you for being a valued partner. We will keep you updated on this patient's progress.

Best regards,

Namrah Quazi PT1360147

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Patient: Tatjana Hocke-Mirzashvili

This document was electronically signed on 02-10-26 at 02:50p by Namrah Quazi PT1360147 Lic# 1360147

Patient: Tatjana Hocke-Mirzashvili
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Phone: (737) 843-7014 Fax: (737) 843-7016
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PLAN OF CARE

Date of Visit: 02-10-26

Referral Source: Martha Pyron
Phone: (512) 676-2500
Fax: (512) 406-7377

Patient Name: Tatjana Hocke-Mirzashvili

Patient Number: TX0458176

Date of Birth: 12-15-1982

Date of Onset: 09-01-25

Certification Period: 02-11-26 - 04-07-26

Diagnosis:

M79.671 Pain in right foot
M79.672 Pain in left foot
M25.562 Pain in left knee
M22.2X2 Patellofemoral disorders, left knee
M54.50 Low back pain, unspecified

ASSESSMENT:

Patient demonstrates varied progress across multiple conditions being treated concurrently. For pelvic floor dysfunction, patient shows good understanding of breathing techniques and muscle isolation exercises, though continues to struggle with proper muscle activation and compensatory patterns. Patient reports difficulty separating pelvic floor contractions from upper abdominal muscle activation, indicating need for continued neuromuscular re-education. Patient demonstrates good compliance with home exercise program and motivation for recovery. Patient would benefit from continued skilled therapy to address neuromuscular re-education needs, manual therapy for tissue mobility, and progressive strengthening while managing multiple concurrent conditions. She is aware of her diagnosis/prognosis. The plans and goals have been developed and discussed with the patient.

REHAB POTENTIAL:

The patient's rehab potential is good.

PLAN:

The patient will continue therapy 2 times per week for 8 weeks, for an additional 16 visits and a total of 36 visits since start of care. This may include the following treatments:

- joint and soft tissue mobilization
- neuromuscular re-education
- therapeutic activities
- Dry Needling
- therapeutic exercises
- therapeutic activities
- gait training
- e-Stim unattended no wound

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Goals	Short-Long	Time Frame	Result	Comment
Patient will be independent in progressive monitored HEP.	Short Term	1 week	Met	
Improve position tolerance of walking on uneven terrain	Long Term	8 weeks	Ongoing	
Improve L single leg stance time to 30 seconds	Long Term	8 weeks	Ongoing	
Pt will stand 30 minutes pain free	Long Term	8 weeks	Partially Met 1-24%	
Patient will be independent with HEP	Short Term	4 weeks		
Patient will demonstrate increase in L/S AROM in all directions w/o pain	Long Term	8 weeks		
Centralization of radicular symptoms	Long Term	8 weeks		
Eliminate post innominate rotation on R Side	Short Term	4 weeks		

Namrah Quazi PT1360147

This document was electronically signed on 02-10-26 at 02:50p by Namrah Quazi PT1360147 Lic# 1360147

This is just for your information - signature not required.

Patient: Tatjana Hocke-Mirzashvili
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Daily Note

Date of Visit: 02-05-26
Patient Name: Hocke-Mirzashvili, Tatjana
Patient #: TX0458176

Therapist: Christobal Saenz
Referring MD: Martha Pyron
 Phone: (512) 676-2500
 Fax: (512) 406-7377

Date of Birth: 12-15-1982
Age: 43
Certification Period: 01-08-26 - 02-18-26

Time In:
Time Out:

Date of Onset: 09-01-25

Diagnosis:

M79.671 Pain in right foot
 M79.672 Pain in left foot
 M25.562 Pain in left knee
 M22.2X2 Patellofemoral disorders, left knee

SUBJECTIVE:

Comments: 2/10 R foot pain currently

2/5/26: Pt reports that she is doing a little bit better. Pt reports that traction helped after her last session. She now feels that there is two types of pain in her foot, one is tingling and the other is a pulling in her foot.

2/2/2026

Pt presents to clinic today reporting that she has been feeling about the same as her last visit. No change in symptoms.

1/29/26: Pt reports that patient reports that she is "feeling fine." Her pain is better. She was pain free until she started driving in her car after her last session. Her routine got thrown off due to the weather and she wasn't able to do her HEP. As a result she feels a bit more pain today.

1/21/26: Pt reports that her pain is around a 3/10 today. It is getting better and feels stable but still bothers her especially when she walks more. McKenzie extension has been improving her symptoms.

Pt is a 42 year old female who reports gradual onset of B heel pain and L knee pain that has been ongoing since 9/2025. Pt reports decreasing foot pain since performing HEP of repeated extension in lying more. Pt reports feeling 50% better overall with pain shifting from heel pad to back of heel and outside of feet. Pt reports a significant decrease in B heel pain since initiating her HEP more. Pt continues to decline dry needling directly to feet.

Contraindications:

- None

OBJECTIVE:

NT this visit.

ACTIVITY LOG:

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Time Based Activities	Comment	Status	Sets	Reps	Weight	Time	Done Today	Special	CPT
Repeated extension in lying	McKenzie extension level 4 (press up on hands) Centralizes symptoms, causes some low back tension that diminishes quickly	Active				6	Yes		Neuro Re-Ed (97112) - [B02]
Joint Mobilization	PA lumbar glides	Active				8	Yes		Neuro Re-Ed (97112) - [B02]
Patient Education	importance of core and glute/hamstring strengthening, as well as stretching and mobility exercises for the thoracic and lumbar spine, as well as hip flexors.	Active				8	Yes		Therapeutic Activity (97530) - [B12]
Heel Raise	X 30	Active				6	Yes		Therapeutic Activity (97530) - [B12]
Bridge		Active				4	Yes		Therapeutic Activity (97530) - [B12]
Pelvic Floor Exercises		Active				4	Yes		Neuro Re-Ed (97112) - [B02]
Posterior Pelvic Tilt		Active				4	Yes		Neuro Re-Ed (97112) - [B02]

ASSESSMENT:

Patient consents to treatment plan and goals and gives verbal informed consent. 2/5/26: Pt with reproduction of her pain with palpation of the Achilles tendon. With palpation she has signs and symptoms consistent with diastasis recti with pain reproduction with abdominal distention.

2/2/2026

The tx session began with cobra pose, followed by soft tissue mobilization the the thoracic and lumbar spine. Pt reported increased tenderness more distally along the lumbar spine. Childs pose and open books were performed to facilitate tissue extensibility of the thoracic and lumbar spine. Pt reported feeling decreased symptoms in bilateral feet after the tx session. Pt education provided on discussing low back findings with physician and whether or not imaging would be recommended. Pt's last MRI of the spine was performed 2 years ago.

1/29/26: Pt's back and feet symptoms continue to respond to McKenzie extension interventions. Her knee pain does improve with tibial IR mobilization, indicating mechanical left knee pain.

1/21/26: Today, the patient's foot pain, numbness, and weakness improves with lumbar PA glides as well as with myofascial release of the piriformis. These signs and symptoms are consistent with neurological origin of symptoms. At next visit, plan to continue to address lumbar radicular symptoms and work on improving core and hip strength.

Pt is a 42 year old female who reports gradual onset of B heel pain and L knee pain that has been ongoing since 9/2025. Pt demonstrates decreased B heel pain that is indicative of a relevant lumbar component as demonstrated by decreased B heel pain from a 6/10 to a 2/10 with pain moving from heel to just above calcaneus. Pt continues to report occasional numbness and tingling in feet with prolonged sitting.

Patient: Tatjana Hocke-Mirzashvili

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Daily Note

Date of Visit: 02-26-26
Patient Name: Hocke-Mirzashvili, Tatjana
Patient #: TX0458176

Therapist: Christobal Saenz
Referring MD: Martha Pyron
Phone: (512) 676-2500
Fax: (512) 406-7377

Date of Birth: 12-15-1982
Age: 43
Certification Period: 02-11-26 - 04-07-26

Time In:
Time Out:

Date of Onset: 09-01-25

Diagnosis:

M79.671 Pain in right foot
M79.672 Pain in left foot
M25.562 Pain in left knee
M22.2X2 Patellofemoral disorders, left knee
M54.50 Low back pain, unspecified

SUBJECTIVE:

Comments: 2/10 R foot pain currently

2/26/26: Pt reports that her right ankle pain has been worse than usual. Overall however, her heel and back pain have been doing better, not quite normal but getting close.

2/24: Patient states that her pain hovers around 1/10 and sometimes its also gone which feels very weird as she is so used to the pain. She is doing her HEP everyday.

2/19/26: Pt reports that her feet are feeling better. She has less issues at rest, but standing still bothers her. She does have a headache, but feels that that is not related.

2/17: Patient went to Zoo over the weekend and her feet hurt but she is doing the exercises. She feels good with exercises. R heel pain 2/10

Patient saw MD last week after talking to her therapists. X ray was taken which showed she has arthritis and disc herniation but PCP doesn't think patient needs new imaging as they don't anticipate big changes in it. Patient was given referral for her lumbar spine. Patient reports being "good for a while" initially but experienced a flare-up after increasing exercise activities over the summer, including swimming and yoga. Patient notes difficulty isolating pelvic floor muscles during breathing exercises, with tendency to activate upper abdominal muscles instead. Sleep quality is affected, requiring soft padding for comfort. Patient demonstrates understanding of exercise modifications and reports compliance with wall stretches when feeling stiff.

Contraindications:

- None

ACTIVITY LOG:

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Time Based Activities	Comment	Status	Sets	Reps	Weight	Time	Done Today	Special	CPT
Calf stretch on step						6	Yes		Neuro Re-Ed (97112) - [B02]
One leg bridge						4	Yes		Therapeutic Activity (97530) - [B12]
Myofascial Release	Calf/ soleus medial and lateral	Active				20	Yes		Neuro Re-Ed (97112) - [B02]
Patient education	ergonomics for cycling	Active				5	Yes		Therapeutic Activity (97530) - [B12]
dead bugs		Active				5	Yes		Neuro Re-Ed (97112) - [B02]

ASSESSMENT:

Patient consents to treatment plan and goals and gives verbal informed consent. 2/26/26: Pt's right ankle pain is reproduced with palpation of the soleus and peroneals and responds well to myofascial release. She has good forms with dead bugs but rapid fatigue. Her snapping feeling reduces with cues for increased knee flexion during exercise.

2/14: Plantar fascia and peroneal release with gua sha performed as patient c/o soreness and tightness. Taping for heel stabilization done today with care instructions given to patient. Dead bugs were added today and patient was appropriately challenged with them. She c/o snapping feeling in her R hip flexor area so hip flexor stretch was added as well.

2/19/26: Pt with improvements in her soft tissue irritability and core strength today.

2/17/26: Manual lumbar traction performed and patient tolerated it well. MFR performed on glutes/piriformis to release tightness.

2/12/26: Pt with improvements in her core strength and motor control.

2/5/26: Pt with reproduction of her pain with palpation of the Achilles tendon. With palpation she has signs and symptoms consistent with diastasis recti with pain reproduction with abdominal distention.

2/2/2026

The tx session began with cobra pose, followed by soft tissue mobilization the the thoracic and lumbar spine. Pt reported increased tenderness more distally along the lumbar spine. Childs pose and open books were performed to facilitate tissue extensibility of the thoracic and lumbar spine. Pt reported feeling decreased symptoms in bilateral feet after the tx session. Pt education provided on discussing low back findings with physician and whether or not imaging would be recommended. Pt's last MRI of the spine was performed 2 years ago.

1/29/26: Pt's back and feet symptoms continue to respond to McKenzie extension interventions. Her knee pain does improve with tibial IR mobilization, indicating mechanical left knee pain.

1/21/26: Today, the patient's foot pain, numbness, and weakness improves with lumbar PA glides as well as with myofascial release of the piriformis. These signs and symptoms are consistent with neurological origin of symptoms. At next visit, plan to continue to address lumbar radicular symptoms and work on improving core and hip strength.

Pt is a 42 year old female who reports gradual onset of B heel pain and L knee pain that has been ongoing since 9/2025. Pt demonstrates decreased B heel pain that is indicative of a relevant lumbar component as demonstrated by decreased B heel pain from a 6/10 to a 2/10 with pain moving from heel to just above calcaneus. Pt continues to report occasional numbness and tingling in feet with prolonged sitting.

Goals	Short-Long	Time Frame	Result	Comment
Patient will be independent in progressive monitored HEP.	Short Term	1 week	Met	
Improve position tolerance of walking on uneven terrain	Long Term	8 weeks	Ongoing	
Improve L single leg stance time to 30 seconds	Long Term	8 weeks	Ongoing	
Pt will stand 30 minutes pain free	Long Term	8 weeks	Partially Met 1-24%	
Patient will be independent with HEP	Short Term	4 weeks		

Patient: Tatjana Hocke-Mirzashvili

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Goals	Short-Long	Time Frame	Result	Comment
Patient will demonstrate increase in L/S AROM in all directions w/o pain	Long Term	8 weeks		
Centralization of radicular symptoms	Long Term	8 weeks		
Eliminate post innominate rotation on R Side	Short Term	4 weeks		

PLAN:

Tatjana will continue therapy as prescribed. This is visit # 25.

Best regards,

Christobal Saenz PT

This document was electronically signed on 02-26-26 at 09:50a by Christobal Saenz PT Lic# TX48758PT



Daily Note

Date of Visit: 02-24-26
Patient Name: Hocke-Mirzashvili, Tatjana
Patient #: TX0458176

Therapist: Namrah Quazi
Referring MD: Martha Pyron
 Phone: (512) 676-2500
 Fax: (512) 406-7377

Date of Birth: 12-15-1982
Age: 43
Certification Period: 02-11-26 - 04-07-26

Time In:
Time Out:

Date of Onset: 09-01-25

Diagnosis:

M79.671 Pain in right foot
 M79.672 Pain in left foot
 M25.562 Pain in left knee
 M22.2X2 Patellofemoral disorders, left knee
 M54.50 Low back pain, unspecified

SUBJECTIVE:

Comments: 2/10 R foot pain currently

2/24: Patient states that her pain hovers around 1/10 and sometimes its also gone which feels very weird as she is so used to the pain. She is doing her HEP everyday.

2/19/26: Pt reports that her feet are feeling better. She has less issues at rest, but standing still bothers her. She does have a headache, but feels that that is not related.

2/17: Patient went to Zoo over the weekend and her feet hurt but she is doing the exercises. She feels good with exercises. R heel pain 2/10

Patient saw MD last week after talking to her therapists. X ray was taken which showed she has arthritis and disc herniation but PCP doesn't think patient needs new imaging as they don't anticipate big changes in it. Patient was given referral for her lumbar spine. Patient reports being "good for a while" initially but experienced a flare-up after increasing exercise activities over the summer, including swimming and yoga. Patient notes difficulty isolating pelvic floor muscles during breathing exercises, with tendency to activate upper abdominal muscles instead. Sleep quality is affected, requiring soft padding for comfort. Patient demonstrates understanding of exercise modifications and reports compliance with wall stretches when feeling stiff.

Contraindications:

- None

ACTIVITY LOG:

Time Based Activities	Comment	Status	Sets	Reps	Weight	Time	Done Today	Special	CPT
Myofascial Release	piriformis bil	Active				4	Yes		Neuro Re-Ed (97112) - [B02]

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Time Based Activities	Comment	Status	Sets	Reps	Weight	Time	Done Today	Special	CPT
hip flexor stretch	2x30"	Active				4	Yes		Neuro Re-Ed (97112) - [B02]
Joint Mobilization	PA lumbar glides/ SI joint AP and inferior glide R side	Active				5	Yes		Neuro Re-Ed (97112) - [B02]
Patient education		Active				6	Yes		Therapeutic Activity (97530) - [B12]
Taping	Heel support	Active				6	Yes		Neuro Re-Ed (97112) - [B02]
dead bugs		Active				5	Yes		Neuro Re-Ed (97112) - [B02]
Soft Tissue Mobilization	R heel and plantar fascia with gua sha release	Active				10	Yes		Neuro Re-Ed (97112) - [B02]

ASSESSMENT:

Patient consents to treatment plan and goals and gives verbal informed consent. 2/14: Plantarfascia and peroneal release with gua sha performed as patient c/o soreness and tightness. Taping for heel stabilization done today with care instructions given to patient. Dead bugs were added today and patient was appropriately challenged with them. She c/o snapping feeling in her R hip flexor area so hip flexor stretch was added as well.

2/19/26: Pt with improvements in her soft tissue irritability and core strength today.

2/17/26: Manual lumbar traction performed and patient tolerated it well. MFR performed on glutes/piriformis to release tightness.

2/12/26: Pt with improvements in her core strength and motor control.

2/5/26: Pt with reproduction of her pain with palpation of the Achilles tendon. With palpation she has signs and symptoms consistent with diastasis recti with pain reproduction with abdominal distention.

2/2/2026

The tx session began with cobra pose, followed by soft tissue mobilization the the thoracic and lumbar spine. Pt reported increased tenderness more distally along the lumbar spine. Childs pose and open books were performed to facilitate tissue extensibility of the thoracic and lumbar spine. Pt reported feeling decreased symptoms in bilateral feet after the tx session. Pt education provided on discussing low back findings with physician and whether or not imaging would be recommended. Pt's last MRI of the spine was performed 2 years ago.

1/29/26: Pt's back and feet symptoms continue to respond to McKenzie extension interventions. Her knee pain does improve with tibial IR mobilization, indicating mechanical left knee pain.

1/21/26: Today, the patient's foot pain, numbness, and weakness improves with lumbar PA glides as well as with myofascial release of the piriformis. These signs and symptoms are consistent with neurological origin of symptoms. At next visit, plan to continue to address lumbar radicular symptoms and work on improving core and hip strength.

Pt is a 42 year old female who reports gradual onset of B heel pain and L knee pain that has been ongoing since 9/2025. Pt demonstrates decreased B heel pain that is indicative of a relevant lumbar component as demonstrated by decreased B heel pain from a 6/10 to a 2/10 with pain moving from heel to just above calcaneus. Pt continues to report occasional numbness and tingling in feet with prolonged sitting.

Goals	Short-Long	Time Frame	Result	Comment
Patient will be independent in progressive monitored HEP.	Short Term	1 week	Met	
Improve position tolerance of walking on uneven terrain	Long Term	8 weeks	Ongoing	
Improve L single leg stance time to 30 seconds	Long Term	8 weeks	Ongoing	
Pt will stand 30 minutes pain free	Long Term	8 weeks	Partially Met	

Patient: Tatjana Hocke-Mirzashvili

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Goals	Short-Long	Time Frame	Result	Comment
			1-24%	
Patient will be independent with HEP	Short Term	4 weeks		
Patient will demonstrate increase in L/S AROM in all directions w/o pain	Long Term	8 weeks		
Centralization of radicular symptoms	Long Term	8 weeks		
Eliminate post innominate rotation on R Side	Short Term	4 weeks		

PLAN:

Tatjana will continue therapy as prescribed. This is visit # 24.

Best regards,

Namrah Quazi PT1360147

This document was electronically signed on 02-24-26 at 09:53a by Namrah Quazi PT1360147 Lic# 1360147



Daily Note

Date of Visit: 02-19-26
Patient Name: Hocke-Mirzashvili, Tatjana
Patient #: TX0458176

Therapist: Christobal Saenz
Referring MD: Martha Pyron
 Phone: (512) 676-2500
 Fax: (512) 406-7377

Date of Birth: 12-15-1982
Age: 43
Certification Period: 02-11-26 - 04-07-26

Time In:
Time Out:

Date of Onset: 09-01-25

Diagnosis:

M79.671 Pain in right foot
 M79.672 Pain in left foot
 M25.562 Pain in left knee
 M22.2X2 Patellofemoral disorders, left knee
 M54.50 Low back pain, unspecified

SUBJECTIVE:

Comments: 2/10 R foot pain currently

2/19/26: Pt reports that her feet are feeling better. She has less issues at rest, but standing still bothers her. She does have a headache, but feels that that is not related.

2/17: Patient went to Zoo over the weekend and her feet hurt but she is doing the exercises. She feels good with exercises. R heel pain 2/10

Patient saw MD last week after talking to her therapists. X ray was taken which showed she has arthritis and disc herniation but PCP doesn't think patient needs new imaging as they don't anticipate big changes in it. Patient was given referral for her lumbar spine. Patient reports being "good for a while" initially but experienced a flare-up after increasing exercise activities over the summer, including swimming and yoga. Patient notes difficulty isolating pelvic floor muscles during breathing exercises, with tendency to activate upper abdominal muscles instead. Sleep quality is affected, requiring soft padding for comfort. Patient demonstrates understanding of exercise modifications and reports compliance with wall stretches when feeling stiff.

Contraindications:

- None

ACTIVITY LOG:

Time Based Activities	Comment	Status	Sets	Reps	Weight	Time	Done Today	Special	CPT
Paloff Press						6	Yes		Therapeutic Activity (97530) - [B12]

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Time Based Activities	Comment	Status	Sets	Reps	Weight	Time	Done Today	Special	CPT
Bridge	3#	Active				4	Yes		Therapeutic Activity (97530) - [B12]
Braced Reverse Marching						6	Yes		Therapeutic Activity (97530) - [B12]
Repeated extension in lying	McKenzie extension level 4 (press up on hands) with belt	Active				4	Yes		Neuro Re-Ed (97112) - [B02]
Posterior Pelvic Tilt		Active				4	Yes		Neuro Re-Ed (97112) - [B02]
Joint Mobilization	PA lumbar glides/ SI joint AP and inferior glide R side	Active				12	Yes		Neuro Re-Ed (97112) - [B02]
Patient education		Active				4	Yes		Therapeutic Activity (97530) - [B12]

ASSESSMENT:

Patient consents to treatment plan and goals and gives verbal informed consent. 2/19/26: Pt with improvements in her soft tissue irritability and core strength today.

2/17/26: Manual lumbar traction performed and patient tolerated it well. MFR performed on glutes/piriformis to release tightness.

2/12/26: Pt with improvements in her core strength and motor control.

2/5/26: Pt with reproduction of her pain with palpation of the Achilles tendon. With palpation she has signs and symptoms consistent with diastasis recti with pain reproduction with abdominal distention.

2/2/2026

The tx session began with cobra pose, followed by soft tissue mobilization the the thoracic and lumbar spine. Pt reported increased tenderness more distally along the lumbar spine. Childs pose and open books were performed to facilitate tissue extensibility of the thoracic and lumbar spine. Pt reported feeling decreased symptoms in bilateral feet after the tx session. Pt education provided on discussing low back findings with physician and whether or not imaging would be recommended. Pt's last MRI of the spine was performed 2 years ago.

1/29/26: Pt's back and feet symptoms continue to respond to McKenzie extension interventions. Her knee pain does improve with tibial IR mobilization, indicating mechanical left knee pain.

1/21/26: Today, the patient's foot pain, numbness, and weakness improves with lumbar PA glides as well as with myofascial release of the piriformis. These signs and symptoms are consistent with neurological origin of symptoms. At next visit, plan to continue to address lumbar radicular symptoms and work on improving core and hip strength.

Pt is a 42 year old female who reports gradual onset of B heel pain and L knee pain that has been ongoing since 9/2025. Pt demonstrates decreased B heel pain that is indicative of a relevant lumbar component as demonstrated by decreased B heel pain from a 6/10 to a 2/10 with pain moving from heel to just above calcaneus. Pt continues to report occasional numbness and tingling in feet with prolonged sitting.

Goals	Short-Long	Time Frame	Result	Comment
Patient will be independent in progressive monitored HEP.	Short Term	1 week	Met	
Improve position tolerance of walking on uneven terrain	Long Term	8 weeks	Ongoing	
Improve L single leg stance time to 30 seconds	Long Term	8 weeks	Ongoing	
Pt will stand 30 minutes pain free	Long Term	8 weeks	Partially Met	

Patient: Tatjana Hocke-Mirzashvili

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Phone: (737) 843-7014 Fax: (737) 843-7016

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Goals	Short-Long	Time Frame	Result	Comment
			1-24%	
Patient will be independent with HEP	Short Term	4 weeks		
Patient will demonstrate increase in L/S AROM in all directions w/o pain	Long Term	8 weeks		
Centralization of radicular symptoms	Long Term	8 weeks		
Eliminate post innominate rotation on R Side	Short Term	4 weeks		

PLAN:

Tatjana will continue therapy as prescribed. This is visit # 23.

Best regards,

Christobal Saenz PT

This document was electronically signed on 02-19-26 at 10:35a by Christobal Saenz PT Lic# TX48758PT



Daily Note

Date of Visit: 02-17-26
Patient Name: Hocke-Mirzashvili, Tatjana
Patient #: TX0458176

Therapist: Namrah Quazi
Referring MD: Martha Pyron
 Phone: (512) 676-2500
 Fax: (512) 406-7377

Date of Birth: 12-15-1982
Age: 43
Certification Period: 02-11-26 - 04-07-26

Time In:
Time Out:

Date of Onset: 09-01-25

Diagnosis:

M79.671 Pain in right foot
 M79.672 Pain in left foot
 M25.562 Pain in left knee
 M22.2X2 Patellofemoral disorders, left knee
 M54.50 Low back pain, unspecified

SUBJECTIVE:

Comments: 2/10 R foot pain currently

2/17: Patient went to Zoo over the weekend and her feet hurt but she is doing the exercises. She feels good with exercises. R heel pain 2/10

Patient saw MD last week after talking to her therapists. X ray was taken which showed she has arthritis and disc herniation but PCP doesn't think patient needs new imaging as they don't anticipate big changes in it. Patient was given referral for her lumbar spine. Patient reports being "good for a while" initially but experienced a flare-up after increasing exercise activities over the summer, including swimming and yoga. Patient notes difficulty isolating pelvic floor muscles during breathing exercises, with tendency to activate upper abdominal muscles instead. Sleep quality is affected, requiring soft padding for comfort. Patient demonstrates understanding of exercise modifications and reports compliance with wall stretches when feeling stiff.

Contraindications:

- None

ACTIVITY LOG:

Time Based Activities	Comment	Status	Sets	Reps	Weight	Time	Done Today	Special	CPT
Lumbar and SI glides	R PA and inferior SI glides; PA glides L/S globally	Active				8	Yes		Neuro Re-Ed (97112) - [B02]
Myofascial	piriformis bil	Active				10	Yes		Neuro Re-Ed

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Time Based Activities	Comment	Status	Sets	Reps	Weight	Time	Done Today	Special	CPT
Release									(97112) - [B02]
Repeated extension in lying	McKenzie extension level 4 (press up on hands) with belt	Active				4	Yes		Neuro Re-Ed (97112) - [B02]
Posterior Pelvic Tilt		Active				4	Yes		Neuro Re-Ed (97112) - [B02]
Manual lumbar tracti		Active				6	Yes		Neuro Re-Ed (97112) - [B02]
Joint Mobilization	PA lumbar glides/ SI joint AP and inferior glide R side	Active				12	Yes		Neuro Re-Ed (97112) - [B02]

ASSESSMENT:

Patient consents to treatment plan and goals and gives verbal informed consent. 2/17/26: Manual lumbar traction performed and patient tolerated it well. MFR performed on glutes/piriformis to release tightness.

2/12/26: Pt with improvements in her core strength and motor control.

2/5/26: Pt with reproduction of her pain with palpation of the Achilles tendon. With palpation she has signs and symptoms consistent with diastasis recti with pain reproduction with abdominal distention.

2/2/2026

The tx session began with cobra pose, followed by soft tissue mobilization the the thoracic and lumbar spine. Pt reported increased tenderness more distally along the lumbar spine. Childs pose and open books were performed to facilitate tissue extensibility of the thoracic and lumbar spine. Pt reported feeling decreased symptoms in bilateral feet after the tx session. Pt education provided on discussing low back findings with physician and whether or not imaging would be recommended. Pt's last MRI of the spine was performed 2 years ago.

1/29/26: Pt's back and feet symptoms continue to respond to McKenzie extension interventions. Her knee pain does improve with tibial IR mobilization, indicating mechanical left knee pain.

1/21/26: Today, the patient's foot pain, numbness, and weakness improves with lumbar PA glides as well as with myofascial release of the piriformis. These signs and symptoms are consistent with neurological origin of symptoms. At next visit, plan to continue to address lumbar radicular symptoms and work on improving core and hip strength.

Pt is a 42 year old female who reports gradual onset of B heel pain and L knee pain that has been ongoing since 9/2025. Pt demonstrates decreased B heel pain that is indicative of a relevant lumbar component as demonstrated by decreased B heel pain from a 6/10 to a 2/10 with pain moving from heel to just above calcaneus. Pt continues to report occasional numbness and tingling in feet with prolonged sitting.

Goals	Short-Long	Time Frame	Result	Comment
Patient will be independent in progressive monitored HEP.	Short Term	1 week	Met	
Improve position tolerance of walking on uneven terrain	Long Term	8 weeks	Ongoing	
Improve L single leg stance time to 30 seconds	Long Term	8 weeks	Ongoing	
Pt will stand 30 minutes pain free	Long Term	8 weeks	Partially Met 1-24%	
Patient will be independent with HEP	Short Term	4 weeks		
Patient will demonstrate increase in L/S AROM in all directions w/o pain	Long Term	8 weeks		
Centralization of radicular symptoms	Long Term	8 weeks		
Eliminate post innominate rotation on R Side	Short Term	4 weeks		

Patient: Tatjana Hocke-Mirzashvili

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Phone: (737) 843-7014 Fax: (737) 843-7016

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PLAN:

Tatjana will continue therapy as prescribed. This is visit # 22.

Best regards,

Namrah Quazi PT1360147

This document was electronically signed on 02-17-26 at 11:19a by Namrah Quazi PT1360147 Lic# 1360147



Daily Note

Date of Visit: 02-12-26
Patient Name: Hocke-Mirzashvili, Tatjana
Patient #: TX0458176

Therapist: Christobal Saenz
Referring MD: Martha Pyron
 Phone: (512) 676-2500
 Fax: (512) 406-7377

Date of Birth: 12-15-1982
Age: 43
Certification Period: 02-11-26 - 04-07-26

Time In:
Time Out:

Date of Onset: 09-01-25

Diagnosis:

M79.671 Pain in right foot
 M79.672 Pain in left foot
 M25.562 Pain in left knee
 M22.2X2 Patellofemoral disorders, left knee
 M54.50 Low back pain, unspecified

SUBJECTIVE:

Comments: 2/10 R foot pain currently

2/12/26: Pt reports that she is feeling overall good. She has a new prescription for her back. Her physician diagnosed her with a disc issue and arthritis. Her foot pain is around a 2/10 but it was about a 4/10 when she had to do a long drive.

Patient saw MD last week after talking to her therapists. X ray was taken which showed she has arthritis and disc herniation but PCP doesn't think patient needs new imaging as they don't anticipate big changes in it. Patient was given referral for her lumbar spine. Patient reports being "good for a while" initially but experienced a flare-up after increasing exercise activities over the summer, including swimming and yoga. Patient notes difficulty isolating pelvic floor muscles during breathing exercises, with tendency to activate upper abdominal muscles instead. Sleep quality is affected, requiring soft padding for comfort. Patient demonstrates understanding of exercise modifications and reports compliance with wall stretches when feeling stiff.

Contraindications:

- None

ACTIVITY LOG:

Time Based Activities	Comment	Status	Sets	Reps	Weight	Time	Done Today	Special	CPT
Isometric Crunch		Active				6	Yes		Therapeutic Activity (97530) - [B12]
Braced Reverse Marching						6	Yes		Therapeutic Activity (97530) -

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Time Based Activities	Comment	Status	Sets	Reps	Weight	Time	Done Today	Special	CPT
									[B12]
Repeated extension in lying	McKenzie extension level 4 (press up on hands) with belt	Active				8	Yes		Neuro Re-Ed (97112) - [B02]
Bridge		Active				4	Yes		Therapeutic Activity (97530) - [B12]
Posterior Pelvic Tilt		Active				4	Yes		Neuro Re-Ed (97112) - [B02]
Joint Mobilization	PA lumbar glides/ SI joint AP and inferior glide R side	Active				12	Yes		Neuro Re-Ed (97112) - [B02]

ASSESSMENT:

Patient consents to treatment plan and goals and gives verbal informed consent. 2/12/26: Pt with improvements in her core strength and motor control.

2/5/26: Pt with reproduction of her pain with palpation of the Achilles tendon. With palpation she has signs and symptoms consistent with diastasis recti with pain reproduction with abdominal distention.

2/2/2026

The tx session began with cobra pose, followed by soft tissue mobilization the the thoracic and lumbar spine. Pt reported increased tenderness more distally along the lumbar spine. Childs pose and open books were performed to facilitate tissue extensibility of the thoracic and lumbar spine. Pt reported feeling decreased symptoms in bilateral feet after the tx session. Pt education provided on discussing low back findings with physician and whether or not imaging would be recommended. Pt's last MRI of the spine was performed 2 years ago.

1/29/26: Pt's back and feet symptoms continue to respond to McKenzie extension interventions. Her knee pain does improve with tibial IR mobilization, indicating mechanical left knee pain.

1/21/26: Today, the patient's foot pain, numbness, and weakness improves with lumbar PA glides as well as with myofascial release of the piriformis. These signs and symptoms are consistent with neurological origin of symptoms. At next visit, plan to continue to address lumbar radicular symptoms and work on improving core and hip strength.

Pt is a 42 year old female who reports gradual onset of B heel pain and L knee pain that has been ongoing since 9/2025. Pt demonstrates decreased B heel pain that is indicative of a relevant lumbar component as demonstrated by decreased B heel pain from a 6/10 to a 2/10 with pain moving from heel to just above calcaneus. Pt continues to report occasional numbness and tingling in feet with prolonged sitting.

Goals	Short-Long	Time Frame	Result	Comment
Patient will be independent in progressive monitored HEP.	Short Term	1 week	Met	
Improve position tolerance of walking on uneven terrain	Long Term	8 weeks	Ongoing	
Improve L single leg stance time to 30 seconds	Long Term	8 weeks	Ongoing	
Pt will stand 30 minutes pain free	Long Term	8 weeks	Partially Met 1-24%	
Patient will be independent with HEP	Short Term	4 weeks		
Patient will demonstrate increase in L/S AROM in all directions w/o pain	Long Term	8 weeks		
Centralization of radicular symptoms	Long Term	8 weeks		
Eliminate post innominate rotation on R Side	Short Term	4 weeks		

Patient: Tatjana Hocke-Mirzashvili

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Phone: (737) 843-7014 Fax: (737) 843-7016

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PLAN:

Tatjana will continue therapy as prescribed. This is visit # 21.

Best regards,

Christobal Saenz PT

This document was electronically signed on 02-12-26 at 12:46p by Christobal Saenz PT Lic# TX48758PT



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Progress Note

Date of Visit: 02-10-26
Patient Name: Hocke-Mirzashvili, Tatjana
Patient #: TX0458176

Therapist: Namrah Quazi
Referring MD: Martha Pyron
Phone: (512) 676-2500
Fax: (512) 406-7377

Date of Birth: 12-15-1982
Age: 43
Certification Period: 02-11-26 - 04-07-26

Time In:
Time Out:

Date of Onset: 09-01-25

Diagnosis:

M79.671 Pain in right foot
M79.672 Pain in left foot
M25.562 Pain in left knee
M22.2X2 Patellofemoral disorders, left knee
M54.50 Low back pain, unspecified

SUBJECTIVE:

Presenting Problems:

The patient reports:

- Difficulty ascending/descending stairs, Moderate
- Difficulty walking, Minimal
- Difficulty squatting, Moderate
- Back stiffness, Moderate
- Difficulty isolating pelvic floor muscles, Moderate

Comments: 2/10 R foot pain currently

Functional Status	Prior	Current
Activities of daily living	100%	50%

Contraindications:

- None

Patient saw MD last week after talking to her therapists. X ray was taken which showed she has arthritis and disc herniation but PCP doesn't think patient needs new imaging as they don't anticipate big changes in it. Patient was given referral for her lumbar spine. Patient reports being "good for a while" initially but experienced a flare-up after increasing exercise activities over the summer, including swimming and yoga. Patient notes difficulty isolating pelvic floor muscles during breathing exercises, with tendency to activate upper abdominal muscles instead. Sleep quality is affected, requiring soft padding for comfort. Patient demonstrates understanding of exercise modifications and reports compliance with wall stretches when feeling stiff.

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OBJECTIVE:
LUMBAR EVALUATION

RANGE OF MOTION:

2/10 PN:

- Spinal flexion: Patient reports whole body pain/tightness during forward bending with toes up
- Side bending: Right side bending causes discomfort, left side bending tolerated better
- Spinal rotation: Left rotation causes some discomfort in opposite side, right rotation well tolerated

MANUAL MUSCLE TEST:

Lower Extremity	Initial
MMT	02-10-26
Right Hip Abduction	3
Left Hip Abduction	3

PALPATION:

SI joint: R AGMR

R post innominate rotation

TTP: R SI joint.

TTP: Bil lumbar paraspinals

FUNCTION/OBSERVATION:

c/o pain and sensitivity in bil heels

SPECIAL TESTS:

- Straight leg raise test: Positive on left side with pain reported in left side/back area
 - Left leg straight leg raise with neck flexion: Increased pain reported in neck area
- Slump test: BIL negative (muscle stretch)

JOINT MOBILITY:

L/S PA glides: Hypomobile and painful lower L/S and L5-S1

ACTIVITY LOG:

Time Based Activities	Comment	Status	Sets	Reps	Weight	Time	Done Today	Special	CPT
Reassessment	Lumbar assessment	Active				15	Yes		Therapeutic Activity (97530) - [B12]
Shotgun SI glide	using dowel	Active				3	Yes		Neuro Re-Ed (97112) - [B02]
Myofascial Release	piriformis	Active				5	Yes		Neuro Re-Ed (97112) - [B02]
Joint Mobilization	PA lumbar glides/ SI joint AP and inferior glide R side	Active				6	Yes		Neuro Re-Ed (97112) - [B02]

Patient: Tatjana Hocke-Mirzashvili

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Time Based Activities	Comment	Status	Sets	Reps	Weight	Time	Done Today	Special	CPT
Patient Education	clinical findings/POC/prognosis/all patient questions were answered	Active				12	Yes		Therapeutic Activity (97530) - [B12]
Pelvic Floor Exercises		Active				2	Yes		Neuro Re-Ed (97112) - [B02]

ASSESSMENT:

Patient demonstrates varied progress across multiple conditions being treated concurrently. For pelvic floor dysfunction, patient shows good understanding of breathing techniques and muscle isolation exercises, though continues to struggle with proper muscle activation and compensatory patterns. Patient reports difficulty separating pelvic floor contractions from upper abdominal muscle activation, indicating need for continued neuromuscular re-education. Patient demonstrates good compliance with home exercise program and motivation for recovery. Patient would benefit from continued skilled therapy to address neuromuscular re-education needs, manual therapy for tissue mobility, and progressive strengthening while managing multiple concurrent conditions. The patient's rehab potential is good. The patient's discharge prognosis is good.

The patient appears motivated and is able to perform exercises correctly with verbal cues only. The patient's progress towards goals is good and her tolerance to treatment is good. Patient consents to treatment plan and goals and gives verbal informed consent.

Goals	Short-Long	Time Frame	Result	Comment
Patient will be independent in progressive monitored HEP.	Short Term	1 week	Met	
Improve position tolerance of walking on uneven terrain	Long Term	8 weeks	Ongoing	
Improve L single leg stance time to 30 seconds	Long Term	8 weeks	Ongoing	
Pt will stand 30 minutes pain free	Long Term	8 weeks	Partially Met 1-24%	
Patient will be independent with HEP	Short Term	4 weeks		
Patient will demonstrate increase in L/S AROM in all directions w/o pain	Long Term	8 weeks		
Centralization of radicular symptoms	Long Term	8 weeks		
Eliminate post innominate rotation on R Side	Short Term	4 weeks		

PLAN:

The patient's treatment will include joint and soft tissue mobilization, neuromuscular re-education, therapeutic activities, Dry Needling, therapeutic exercises, therapeutic activities, gait training and e-Stim unattended no wound. The patient will continue therapy 2 times per week for 8 weeks, for an additional 16 visits and a total of 36 visits since start of care. This is visit # 20.

Thank you for being a valued partner. We will keep you updated on this patient's progress.

Best regards,

Namrah Quazi PT1360147

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Patient: Tatjana Hocke-Mirzashvili

This document was electronically signed on 02-10-26 at 02:50p by Namrah Quazi PT1360147 Lic# 1360147

Patient: Tatjana Hocke-Mirzashvili
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PLAN OF CARE

Date of Visit: 02-10-26

Referral Source: Martha Pyron
Phone: (512) 676-2500
Fax: (512) 406-7377

Patient Name: Tatjana Hocke-Mirzashvili

Patient Number: TX0458176

Date of Birth: 12-15-1982

Date of Onset: 09-01-25

Certification Period: 02-11-26 - 04-07-26

Diagnosis:

M79.671 Pain in right foot
M79.672 Pain in left foot
M25.562 Pain in left knee
M22.2X2 Patellofemoral disorders, left knee
M54.50 Low back pain, unspecified

ASSESSMENT:

Patient demonstrates varied progress across multiple conditions being treated concurrently. For pelvic floor dysfunction, patient shows good understanding of breathing techniques and muscle isolation exercises, though continues to struggle with proper muscle activation and compensatory patterns. Patient reports difficulty separating pelvic floor contractions from upper abdominal muscle activation, indicating need for continued neuromuscular re-education. Patient demonstrates good compliance with home exercise program and motivation for recovery. Patient would benefit from continued skilled therapy to address neuromuscular re-education needs, manual therapy for tissue mobility, and progressive strengthening while managing multiple concurrent conditions. She is aware of her diagnosis/prognosis. The plans and goals have been developed and discussed with the patient.

REHAB POTENTIAL:

The patient's rehab potential is good.

PLAN:

The patient will continue therapy 2 times per week for 8 weeks, for an additional 16 visits and a total of 36 visits since start of care. This may include the following treatments:

- joint and soft tissue mobilization
- neuromuscular re-education
- therapeutic activities
- Dry Needling
- therapeutic exercises
- therapeutic activities
- gait training
- e-Stim unattended no wound

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Goals	Short-Long	Time Frame	Result	Comment
Patient will be independent in progressive monitored HEP.	Short Term	1 week	Met	
Improve position tolerance of walking on uneven terrain	Long Term	8 weeks	Ongoing	
Improve L single leg stance time to 30 seconds	Long Term	8 weeks	Ongoing	
Pt will stand 30 minutes pain free	Long Term	8 weeks	Partially Met 1-24%	
Patient will be independent with HEP	Short Term	4 weeks		
Patient will demonstrate increase in L/S AROM in all directions w/o pain	Long Term	8 weeks		
Centralization of radicular symptoms	Long Term	8 weeks		
Eliminate post innominate rotation on R Side	Short Term	4 weeks		

Namrah Quazi PT1360147

This document was electronically signed on 02-10-26 at 02:50p by Namrah Quazi PT1360147 Lic# 1360147

This is just for your information - signature not required.

Patient: Tatjana Hocke-Mirzashvili
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 Phone: (737) 843-7014 Fax: (737) 843-7016
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Daily Note

Date of Visit: 02-05-26
Patient Name: Hocke-Mirzashvili, Tatjana
Patient #: TX0458176

Therapist: Christobal Saenz
Referring MD: Martha Pyron
 Phone: (512) 676-2500
 Fax: (512) 406-7377

Date of Birth: 12-15-1982
Age: 43
Certification Period: 01-08-26 - 02-18-26

Time In:
Time Out:

Date of Onset: 09-01-25

Diagnosis:

M79.671 Pain in right foot
 M79.672 Pain in left foot
 M25.562 Pain in left knee
 M22.2X2 Patellofemoral disorders, left knee

SUBJECTIVE:

Comments: 2/10 R foot pain currently

2/5/26: Pt reports that she is doing a little bit better. Pt reports that traction helped after her last session. She now feels that there is two types of pain in her foot, one is tingling and the other is a pulling in her foot.

2/2/2026

Pt presents to clinic today reporting that she has been feeling about the same as her last visit. No change in symptoms.

1/29/26: Pt reports that patient reports that she is "feeling fine." Her pain is better. She was pain free until she started driving in her car after her last session. Her routine got thrown off due to the weather and she wasn't able to do her HEP. As a result she feels a bit more pain today.

1/21/26: Pt reports that her pain is around a 3/10 today. It is getting better and feels stable but still bothers her especially when she walks more. McKenzie extension has been improving her symptoms.

Pt is a 42 year old female who reports gradual onset of B heel pain and L knee pain that has been ongoing since 9/2025. Pt reports decreasing foot pain since performing HEP of repeated extension in lying more. Pt reports feeling 50% better overall with pain shifting from heel pad to back of heel and outside of feet. Pt reports a significant decrease in B heel pain since initiating her HEP more. Pt continues to decline dry needling directly to feet.

Contraindications:

- None

OBJECTIVE:

NT this visit.

ACTIVITY LOG:

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Time Based Activities	Comment	Status	Sets	Reps	Weight	Time	Done Today	Special	CPT
Repeated extension in lying	McKenzie extension level 4 (press up on hands) Centralizes symptoms, causes some low back tension that diminishes quickly	Active				6	Yes		Neuro Re-Ed (97112) - [B02]
Joint Mobilization	PA lumbar glides	Active				8	Yes		Neuro Re-Ed (97112) - [B02]
Patient Education	importance of core and glute/hamstring strengthening, as well as stretching and mobility exercises for the thoracic and lumbar spine, as well as hip flexors.	Active				8	Yes		Therapeutic Activity (97530) - [B12]
Heel Raise	X 30	Active				6	Yes		Therapeutic Activity (97530) - [B12]
Bridge		Active				4	Yes		Therapeutic Activity (97530) - [B12]
Pelvic Floor Exercises		Active				4	Yes		Neuro Re-Ed (97112) - [B02]
Posterior Pelvic Tilt		Active				4	Yes		Neuro Re-Ed (97112) - [B02]

ASSESSMENT:

Patient consents to treatment plan and goals and gives verbal informed consent. 2/5/26: Pt with reproduction of her pain with palpation of the Achilles tendon. With palpation she has signs and symptoms consistent with diastasis recti with pain reproduction with abdominal distention.

2/2/2026

The tx session began with cobra pose, followed by soft tissue mobilization the the thoracic and lumbar spine. Pt reported increased tenderness more distally along the lumbar spine. Childs pose and open books were performed to facilitate tissue extensibility of the thoracic and lumbar spine. Pt reported feeling decreased symptoms in bilateral feet after the tx session. Pt education provided on discussing low back findings with physician and whether or not imaging would be recommended. Pt's last MRI of the spine was performed 2 years ago.

1/29/26: Pt's back and feet symptoms continue to respond to McKenzie extension interventions. Her knee pain does improve with tibial IR mobilization, indicating mechanical left knee pain.

1/21/26: Today, the patient's foot pain, numbness, and weakness improves with lumbar PA glides as well as with myofascial release of the piriformis. These signs and symptoms are consistent with neurological origin of symptoms. At next visit, plan to continue to address lumbar radicular symptoms and work on improving core and hip strength.

Pt is a 42 year old female who reports gradual onset of B heel pain and L knee pain that has been ongoing since 9/2025. Pt demonstrates decreased B heel pain that is indicative of a relevant lumbar component as demonstrated by decreased B heel pain from a 6/10 to a 2/10 with pain moving from heel to just above calcaneus. Pt continues to report occasional numbness and tingling in feet with prolonged sitting.

Patient: Tatjana Hocke-Mirzashvili

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Phone: (737) 843-7014 Fax: (737) 843-7016

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Goals	Short-Long	Time Frame	Result	Comment
Patient will be independent in progressive monitored HEP.	Short Term	1 week	Met	
Improve position tolerance of walking on uneven terrain	Long Term	8 weeks	Ongoing	
Improve L single leg stance time to 30 seconds	Long Term	8 weeks	Ongoing	
Pt will stand 30 minutes pain free	Long Term	8 weeks	Partially Met 1-24%	

PLAN:

Tatjana will continue therapy as prescribed. This is visit # 19. Continue with current POC.

Best regards,

Christobal Saenz PT

This document was electronically signed on 02-05-26 at 09:54a by Christobal Saenz PT Lic# TX48758PT



Endeavor Physical Therapy
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Daily Note

Date of Visit: 02-02-26
Patient Name: Hocke-Mirzashvili, Tatjana
Patient #: TX0458176

Therapist: Daniel Ciezobka
Referring MD: Martha Pyron
Phone: (512) 676-2500
Fax: (512) 406-7377

Date of Birth: 12-15-1982
Age: 43
Certification Period: 01-08-26 - 02-18-26

Time In:
Time Out:

Date of Onset: 09-01-25

Diagnosis:

M79.671 Pain in right foot
M79.672 Pain in left foot
M25.562 Pain in left knee
M22.2X2 Patellofemoral disorders, left knee

SUBJECTIVE:

Comments: 2/10 R foot pain currently

2/2/2026

Pt presents to clinic today reporting that she has been feeling about the same as her last visit. No change in symptoms.

1/29/26: Pt reports that patient reports that she is "feeling fine." Her pain is better. She was pain free until she started driving in her car after her last session. Her routine got thrown off due to the weather and she wasn't able to do her HEP. As a result she feels a bit more pain today.

1/21/26: Pt reports that her pain is around a 3/10 today. It is getting better and feels stable but still bothers her especially when she walks more. McKenzie extension has been improving her symptoms.

Pt is a 42 year old female who reports gradual onset of B heel pain and L knee pain that has been ongoing since 9/2025. Pt reports decreasing foot pain since performing HEP of repeated extension in lying more. Pt reports feeling 50% better overall with pain shifting from heel pad to back of heel and outside of feet. Pt reports a significant decrease in B heel pain since initiating her HEP more. Pt continues to decline dry needling directly to feet.

Contraindications:

- None

OBJECTIVE:

NT this visit.

ACTIVITY LOG:

Time Based Activities	Comment	Status	Sets	Reps	Weight	Time	Done Today	Special	CPT
Repeated extension in lying	McKenzie extension level 4	Active				6	Yes		Ther-Ex (97110) - [B01]

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Time Based Activities	Comment	Status	Sets	Reps	Weight	Time	Done Today	Special	CPT
	(press up on hands) Centralizes symptoms, causes some low back tension that diminishes quickly								
Child pose	1x15	Active				3	Yes		Ther-Ex (97110) - [B01]
Open Books	1x15 bilaterally	Active				4	Yes		Ther-Ex (97110) - [B01]
Joint Mobilization	PA lumbar glides Left knee tibial IR (not performed today)	Active				6	Yes		Neuro Re-Ed (97112) - [B02]
Soft Tissue Mobilization	TrP lumbar spine Manual lumbar traction	Active				15	Yes		Neuro Re-Ed (97112) - [B02]
Patient Education	importance of core and glute/hamstring strengthening, as well as stretching and mobility exercises for the thoracic and lumbar spine, as well as hip flexors.	Active				8	Yes		Therapeutic Activity (97530) - [B12]

ASSESSMENT:

Patient consents to treatment plan and goals and gives verbal informed consent. 2/2/2026

The tx session began with cobra pose, followed by soft tissue mobilization the the thoracic and lumbar spine. Pt reported increased tenderness more distally along the lumbar spine. Childs pose and open books were performed to facilitate tissue extensibility of the thoracic and lumbar spine. Pt reported feeling decreased symptoms in bilateral feet after the tx session. Pt education provided on discussing low back findings with physician and whether or not imaging would be recommended. Pt's last MRI of the spine was performed 2 years ago.

1/29/26: Pt's back and feet symptoms continue to respond to McKenzie extension interventions. Her knee pain does improve with tibial IR mobilization, indicating mechanical left knee pain.

1/21/26: Today, the patient's foot pain, numbness, and weakness improves with lumbar PA glides as well as with myofascial release of the piriformis. These signs and symptoms are consistent with neurological origin of symptoms. At next visit, plan to continue to address lumbar radicular symptoms and work on improving core and hip strength.

Pt is a 42 year old female who reports gradual onset of B heel pain and L knee pain that has been ongoing since 9/2025. Pt demonstrates decreased B heel pain that is indicative of a relevant lumbar component as demonstrated by decreased B heel pain from a 6/10 to a 2/10 with pain moving from heel to just above calcaneus. Pt continues to report occasional numbness and tingling in feet with prolonged sitting.

Goals	Short-Long	Time Frame	Result	Comment
Patient will be independent in progressive monitored HEP.	Short Term	1 week	Met	
Improve position tolerance of walking on uneven terrain	Long Term	8 weeks	Ongoing	

Patient: Tatjana Hocke-Mirzashvili

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Goals	Short-Long	Time Frame	Result	Comment
Improve L single leg stance time to 30 seconds	Long Term	8 weeks	Ongoing	
Pt will stand 30 minutes pain free	Long Term	8 weeks	Partially Met 1-24%	

PLAN:

Tatjana will continue therapy as prescribed. This is visit # 18. Continue with current POC.

Best regards,

Daniel Ciezobka PT

This document was electronically signed on 02-02-26 at 02:35p by Daniel Ciezobka PT Lic# 1407386

Diane Hodges Popps PT

This document was electronically cosigned on 02-02-26 at 05:36p by Diane Hodges Popps PT Lic# TX1218117PT



Daily Note

Date of Visit: 01-29-26
Patient Name: Hocke-Mirzashvili, Tatjana
Patient #: TX0458176

Therapist: Christobal Saenz
Referring MD: Martha Pyron
 Phone: (512) 676-2500
 Fax: (512) 406-7377

Date of Birth: 12-15-1982
Age: 43
Certification Period: 01-08-26 - 02-18-26

Time In:
Time Out:

Date of Onset: 09-01-25

Diagnosis:

M79.671 Pain in right foot
 M79.672 Pain in left foot
 M25.562 Pain in left knee
 M22.2X2 Patellofemoral disorders, left knee

SUBJECTIVE:

Comments: 2/10 R foot pain currently

1/29/26: Pt reports that patient reports that she is "feeling fine." Her pain is better. She was pain free until she started driving in her car after her last session. Her routine got thrown off due to the weather and she wasn't able to do her HEP. As a result she feels a bit more pain today.

1/21/26: Pt reports that her pain is around a 3/10 today. It is getting better and feels stable but still bothers her especially when she walks more. McKenzie extension has been improving her symptoms.

Pt is a 42 year old female who reports gradual onset of B heel pain and L knee pain that has been ongoing since 9/2025. Pt reports decreasing foot pain since performing HEP of repeated extension in lying more. Pt reports feeling 50% better overall with pain shifting from heel pad to back of heel and outside of feet. Pt reports a significant decrease in B heel pain since initiating her HEP more. Pt continues to decline dry needling directly to feet.

Contraindications:

- None

ACTIVITY LOG:

Time Based Activities	Comment	Status	Sets	Reps	Weight	Time	Done Today	Special	CPT
Patient Education	car ergonomics, posture	Active				8	Yes		Therapeutic Activity (97530) - [B12]
Repeated extension in lying	McKenzie extension level 4 (press up on hands)	Active				8	Yes		Therapeutic Activity (97530) - [B12]

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Time Based Activities	Comment	Status	Sets	Reps	Weight	Time	Done Today	Special	CPT
	Centralizes symptoms, causes some low back tension that diminishes quickly								
Joint Mobilization	PA lumbar glides Left knee tibial IR	Active				15	Yes		Neuro Re-Ed (97112) - [B02]
Lifting Technique		Active				8	Yes		Neuro Re-Ed (97112) - [B02]

ASSESSMENT:

Patient consents to treatment plan and goals and gives verbal informed consent. 1/29/26: Pt's back and feet symptoms continue to respond to McKenzie extension interventions. Her knee pain does improve with tibial IR mobilization, indicating mechanical left knee pain.

1/21/26: Today, the patient's foot pain, numbness, and weakness improves with lumbar PA glides as well as with myofascial release of the piriformis. These signs and symptoms are consistent with neurological origin of symptoms. At next visit, plan to continue to address lumbar radicular symptoms and work on improving core and hip strength.

Pt is a 42 year old female who reports gradual onset of B heel pain and L knee pain that has been ongoing since 9/2025. Pt demonstrates decreased B heel pain that is indicative of a relevant lumbar component as demonstrated by decreased B heel pain from a 6/10 to a 2/10 with pain moving from heel to just above calcaneus. Pt continues to report occasional numbness and tingling in feet with prolonged sitting.

Goals	Short-Long	Time Frame	Result	Comment
Patient will be independent in progressive monitored HEP.	Short Term	1 week	Met	
Improve position tolerance of walking on uneven terrain	Long Term	8 weeks	Ongoing	
Improve L single leg stance time to 30 seconds	Long Term	8 weeks	Ongoing	
Pt will stand 30 minutes pain free	Long Term	8 weeks	Partially Met 1-24%	

PLAN:

Tatjana will continue therapy as prescribed. This is visit # 17.

Best regards,

Christobal Saenz PT

This document was electronically signed on 01-29-26 at 10:03a by Christobal Saenz PT Lic# TX48758PT

Patient: Tatjana Hocke-Mirzashvili

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Daily Note

Date of Visit: 01-21-26
Patient Name: Hocke-Mirzashvili, Tatjana
Patient #: TX0458176

Therapist: Christobal Saenz
Referring MD: Martha Pyron
 Phone: (512) 676-2500
 Fax: (512) 406-7377

Date of Birth: 12-15-1982
Age: 43
Certification Period: 01-08-26 - 02-18-26

Time In:
Time Out:

Date of Onset: 09-01-25

Diagnosis:

M79.671 Pain in right foot
 M79.672 Pain in left foot
 M25.562 Pain in left knee
 M22.2X2 Patellofemoral disorders, left knee

SUBJECTIVE:

Comments: 2/10 R foot pain currently

1/21/26: Pt reports that her pain is around a 3/10 today. It is getting better and feels stable but still bothers her especially when she walks more. McKenzie extension has been improving her symptoms.

Pt is a 42 year old female who reports gradual onset of B heel pain and L knee pain that has been ongoing since 9/2025. Pt reports decreasing foot pain since performing HEP of repeated extension in lying more. Pt reports feeling 50% better overall with pain shifting from heel pad to back of heel and outside of feet. Pt reports a significant decrease in B heel pain since initiating her HEP more. Pt continues to decline dry needling directly to feet.

Contraindications:

- None

ACTIVITY LOG:

Time Based Activities	Comment	Status	Sets	Reps	Weight	Time	Done Today	Special	CPT
Patient Education		Active				8	Yes		Therapeutic Activity (97530) - [B12]
Repeated extension in lying	McKenzie extension level 4 (press up on hands) Centralizes symptoms, causes some low	Active				8	Yes		Therapeutic Activity (97530) - [B12]

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Time Based Activities	Comment	Status	Sets	Reps	Weight	Time	Done Today	Special	CPT
	back tension that diminishes quickly								
Joint Mobilization		Active				12	Yes		Neuro Re-Ed (97112) - [B02]
Myofascial Release	piriformis	Active				8	Yes		Neuro Re-Ed (97112) - [B02]

ASSESSMENT:

Patient consents to treatment plan and goals and gives verbal informed consent. 1/21/26: Today, the patient's foot pain, numbness, and weakness improves with lumbar PA glides as well as with myofascial release of the piriformis. These signs and symptoms are consistent with neurological origin of symptoms. At next visit, plan to continue to address lumbar radicular symptoms and work on improving core and hip strength.

Pt is a 42 year old female who reports gradual onset of B heel pain and L knee pain that has been ongoing since 9/2025. Pt demonstrates decreased B heel pain that is indicative of a relevant lumbar component as demonstrated by decreased B heel pain from a 6/10 to a 2/10 with pain moving from heel to just above calcaneus. Pt continues to report occasional numbness and tingling in feet with prolonged sitting.

Goals	Short-Long	Time Frame	Result	Comment
Patient will be independent in progressive monitored HEP.	Short Term	1 week	Met	
Improve position tolerance of walking on uneven terrain	Long Term	8 weeks	Ongoing	
Improve L single leg stance time to 30 seconds	Long Term	8 weeks	Ongoing	
Pt will stand 30 minutes pain free	Long Term	8 weeks	Partially Met 1-24%	

PLAN:

Tatjana will continue therapy as prescribed. This is visit # 16.

Best regards,

Christobal Saenz PT

This document was electronically signed on 01-21-26 at 11:15a by Christobal Saenz PT Lic# TX48758PT

Patient: Tatjana Hocke-Mirzashvili

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Daily Note

Date of Visit: 01-19-26
Patient Name: Hocke-Mirzashvili, Tatjana
Patient #: TX0458176

Therapist: Vicky Bledsoe
Referring MD: Martha Pyron
 Phone: (512) 676-2500
 Fax: (512) 406-7377

Date of Birth: 12-15-1982
Age: 43
Certification Period: 01-08-26 - 02-18-26

Time In:
Time Out:

Date of Onset: 09-01-25

Diagnosis:

M79.671 Pain in right foot
 M79.672 Pain in left foot
 M25.562 Pain in left knee
 M22.2X2 Patellofemoral disorders, left knee

SUBJECTIVE:

Comments: 2/10 R foot pain currently

Pt is a 42 year old female who reports gradual onset of B heel pain and L knee pain that has been ongoing since 9/2025. Pt reports decreasing foot pain since performing HEP of repeated extension in lying more. Pt reports feeling 50% better overall with pain shifting from heel pad to back of heel and outside of feet. Pt reports a significant decrease in B heel pain since initiating her HEP more. Pt continues to decline dry needling directly to feet.

Contraindications:

- None

ACTIVITY LOG:

Time Based Activities	Comment	Status	Sets	Reps	Weight	Time	Done Today	Special	CPT
treadmill	warm up backwards. speed to pt tolerance with 5% incline	Active				8	Yes		Therapeutic Activity (97530) - [B12]
Manual Therapy	IASTM to R distal lateral heel and fibularis mm	Active				15	Yes		Neuro Re-Ed (97112) - [B02]
Repeated extension in lying	3 X 10	Active				15	Yes		Therapeutic Activity (97530) - [B12]
Gastroc/Soleous Stretch	decline board	Active				4	Yes		Therapeutic Activity (97530) -

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Time Based Activities	Comment	Status	Sets	Reps	Weight	Time	Done Today	Special	CPT
									[B12]

ASSESSMENT:

Patient consents to treatment plan and goals and gives verbal informed consent. Pt is a 42 year old female who reports gradual onset of B heel pain and L knee pain that has been ongoing since 9/2025. Pt demonstrates decreased B heel pain that is indicative of a relevant lumbar component as demonstrated by decreased B heel pain from a 6/10 to a 2/10 with pain moving from heel to just above calcaneus. Pt continues to report occasional numbness and tingling in feet with prolonged sitting.

Goals	Short-Long	Time Frame	Result	Comment
Patient will be independent in progressive monitored HEP.	Short Term	1 week	Met	
Improve position tolerance of walking on uneven terrain	Long Term	8 weeks	Ongoing	
Improve L single leg stance time to 30 seconds	Long Term	8 weeks	Ongoing	
Pt will stand 30 minutes pain free	Long Term	8 weeks	Partially Met 1-24%	

PLAN:

Tatjana will continue therapy as prescribed. This is visit # 15.

Best regards,

Vicky Bledsoe PT

This document was electronically signed on 01-19-26 at 10:28a by Vicky Bledsoe PT Lic# 1098267



Endeavor Physical Therapy
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Daily Note

Date of Visit: 01-14-26
Patient Name: Hocke-Mirzashvili, Tatjana
Patient #: TX0458176

Therapist: Vicky Bledsoe
Referring MD: Martha Pyron
Phone: (512) 676-2500
Fax: (512) 406-7377

Date of Birth: 12-15-1982
Age: 43
Certification Period: 01-08-26 - 02-18-26

Time In:
Time Out:

Date of Onset: 09-01-25

Diagnosis:

M79.671 Pain in right foot
M79.672 Pain in left foot
M25.562 Pain in left knee
M22.2X2 Patellofemoral disorders, left knee

SUBJECTIVE:

Comments: 6/10 R foot pain currently

Pt is a 42 year old female who reports gradual onset of B heel pain and L knee pain that has been ongoing since 9/2025. Pt reports increased R > L heel pain today after sitting in car taking a work meeting. Pt states she was resting on her heels with her ankle dorsiflexed. Pt states she was sittign in this position for 35 minutes.

Contraindications:

- None

ACTIVITY LOG:

Time Based Activities	Comment	Status	Sets	Reps	Weight	Time	Done Today	Special	CPT
Recumbent Bike	warm up	Active				8	Yes		Therapeutic Activity (97530) - [B12]
Manual Therapy	IASTM to R distal lateral heel and fibularis mm	Active				15	Yes		Neuro Re-Ed (97112) - [B02]
Repeated extension in lying	3 X 10	Active				10	Yes		Therapeutic Activity (97530) - [B12]
Gastroc/Soleous Stretch	decline board	Active				4	Yes		Therapeutic Activity (97530) - [B12]
Heel Raise	on decline board X 30	Active				4	Yes		Therapeutic Activity (97530) -

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Time Based Activities	Comment	Status	Sets	Reps	Weight	Time	Done Today	Special	CPT
									[B12]

ASSESSMENT:

Patient consents to treatment plan and goals and gives verbal informed consent. Pt is a 42 year old female who reports gradual onset of B heel pain and L knee pain that has been ongoing since 9/2025. Pt demonstrates decreased pain that is indicative of a relevant lumbar component as demonstrated by decreased B heel pain from a 6/10 to a 2/10 with pain moving from heel to just above calcaneus.

Goals	Short-Long	Time Frame	Result	Comment
Patient will be independent in progressive monitored HEP.	Short Term	1 week	Met	
Improve position tolerance of walking on uneven terrain	Long Term	8 weeks	Ongoing	
Improve L single leg stance time to 30 seconds	Long Term	8 weeks	Ongoing	
Pt will stand 30 minutes pain free	Long Term	8 weeks	Partially Met 1-24%	

PLAN:

Tatjana will continue therapy as prescribed. This is visit # 14.

Best regards,

Vicky Bledsoe PT

This document was electronically signed on 01-14-26 at 10:20a by Vicky Bledsoe PT Lic# 1098267



Endeavor Physical Therapy
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To: Aetna Holista
Fax number: (866) 231-6344

From: Ann Dugger
Fax number: (737) 843-7016

Date: 02-27-26

Regarding : Tatjana Hocke-Mirzashvili
DOB: 12-15-1982

Phone number for follow-up:
(737) 843-7014

Comments: Tatjana Hocke-Mirzashvili

Requested medical records for Tatjana Hocke-Mirzashvili, ID #W236967554

Endeavor Cedar Park
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Cedar Park TX, 786137860

Fax: (737) 843-7016
Phone: (737) 843-7014



Daily Note

Date of Visit: 02-26-26
Patient Name: Hocke-Mirzashvili, Tatjana
Patient #: TX0458176

Therapist: Christobal Saenz
Referring MD: Martha Pyron
 Phone: (512) 676-2500
 Fax: (512) 406-7377

Date of Birth: 12-15-1982
Age: 43
Certification Period: 02-11-26 - 04-07-26

Time In:
Time Out:

Date of Onset: 09-01-25

Diagnosis:

M79.671 Pain in right foot
 M79.672 Pain in left foot
 M25.562 Pain in left knee
 M22.2X2 Patellofemoral disorders, left knee
 M54.50 Low back pain, unspecified

SUBJECTIVE:

Comments: 2/10 R foot pain currently

2/26/26: Pt reports that her right ankle pain has been worse than usual. Overall however, her heel and back pain have been doing better, not quite normal but getting close.

2/24: Patient states that her pain hovers around 1/10 and sometimes its also gone which feels very weird as she is so used to the pain. She is doing her HEP everyday.

2/19/26: Pt reports that her feet are feeling better. She has less issues at rest, but standing still bothers her. She does have a headache, but feels that that is not related.

2/17: Patient went to Zoo over the weekend and her feet hurt but she is doing the exercises. She feels good with exercises. R heel pain 2/10

Patient saw MD last week after talking to her therapists. X ray was taken which showed she has arthritis and disc herniation but PCP doesn't think patient needs new imaging as they don't anticipate big changes in it. Patient was given referral for her lumbar spine. Patient reports being "good for a while" initially but experienced a flare-up after increasing exercise activities over the summer, including swimming and yoga. Patient notes difficulty isolating pelvic floor muscles during breathing exercises, with tendency to activate upper abdominal muscles instead. Sleep quality is affected, requiring soft padding for comfort. Patient demonstrates understanding of exercise modifications and reports compliance with wall stretches when feeling stiff.

Contraindications:

- None

ACTIVITY LOG:

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Time Based Activities	Comment	Status	Sets	Reps	Weight	Time	Done Today	Special	CPT
Calf stretch on step						6	Yes		Neuro Re-Ed (97112) - [B02]
One leg bridge						4	Yes		Therapeutic Activity (97530) - [B12]
Myofascial Release	Calf/ soleus medial and lateral	Active				20	Yes		Neuro Re-Ed (97112) - [B02]
Patient education	ergonomics for cycling	Active				5	Yes		Therapeutic Activity (97530) - [B12]
dead bugs		Active				5	Yes		Neuro Re-Ed (97112) - [B02]

ASSESSMENT:

Patient consents to treatment plan and goals and gives verbal informed consent. 2/26/26: Pt's right ankle pain is reproduced with palpation of the soleus and peroneals and responds well to myofascial release. She has good forms with dead bugs but rapid fatigue. Her snapping feeling reduces with cues for increased knee flexion during exercise.

2/14: Plantar fascia and peroneal release with gua sha performed as patient c/o soreness and tightness. Taping for heel stabilization done today with care instructions given to patient. Dead bugs were added today and patient was appropriately challenged with them. She c/o snapping feeling in her R hip flexor area so hip flexor stretch was added as well.

2/19/26: Pt with improvements in her soft tissue irritability and core strength today.

2/17/26: Manual lumbar traction performed and patient tolerated it well. MFR performed on glutes/piriformis to release tightness.

2/12/26: Pt with improvements in her core strength and motor control.

2/5/26: Pt with reproduction of her pain with palpation of the Achilles tendon. With palpation she has signs and symptoms consistent with diastasis recti with pain reproduction with abdominal distention.

2/2/2026

The tx session began with cobra pose, followed by soft tissue mobilization the the thoracic and lumbar spine. Pt reported increased tenderness more distally along the lumbar spine. Childs pose and open books were performed to facilitate tissue extensibility of the thoracic and lumbar spine. Pt reported feeling decreased symptoms in bilateral feet after the tx session. Pt education provided on discussing low back findings with physician and whether or not imaging would be recommended. Pt's last MRI of the spine was performed 2 years ago.

1/29/26: Pt's back and feet symptoms continue to respond to McKenzie extension interventions. Her knee pain does improve with tibial IR mobilization, indicating mechanical left knee pain.

1/21/26: Today, the patient's foot pain, numbness, and weakness improves with lumbar PA glides as well as with myofascial release of the piriformis. These signs and symptoms are consistent with neurological origin of symptoms. At next visit, plan to continue to address lumbar radicular symptoms and work on improving core and hip strength.

Pt is a 42 year old female who reports gradual onset of B heel pain and L knee pain that has been ongoing since 9/2025. Pt demonstrates decreased B heel pain that is indicative of a relevant lumbar component as demonstrated by decreased B heel pain from a 6/10 to a 2/10 with pain moving from heel to just above calcaneus. Pt continues to report occasional numbness and tingling in feet with prolonged sitting.

Goals	Short-Long	Time Frame	Result	Comment
Patient will be independent in progressive monitored HEP.	Short Term	1 week	Met	
Improve position tolerance of walking on uneven terrain	Long Term	8 weeks	Ongoing	
Improve L single leg stance time to 30 seconds	Long Term	8 weeks	Ongoing	
Pt will stand 30 minutes pain free	Long Term	8 weeks	Partially Met 1-24%	
Patient will be independent with HEP	Short Term	4 weeks		

Patient: Tatjana Hocke-Mirzashvili

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Goals	Short-Long	Time Frame	Result	Comment
Patient will demonstrate increase in L/S AROM in all directions w/o pain	Long Term	8 weeks		
Centralization of radicular symptoms	Long Term	8 weeks		
Eliminate post innominate rotation on R Side	Short Term	4 weeks		

PLAN:

Tatjana will continue therapy as prescribed. This is visit # 25.

Best regards,

Christobal Saenz PT

This document was electronically signed on 02-26-26 at 09:50a by Christobal Saenz PT Lic# TX48758PT



Daily Note

Date of Visit: 02-24-26
Patient Name: Hocke-Mirzashvili, Tatjana
Patient #: TX0458176

Therapist: Namrah Quazi
Referring MD: Martha Pyron
 Phone: (512) 676-2500
 Fax: (512) 406-7377

Date of Birth: 12-15-1982
Age: 43
Certification Period: 02-11-26 - 04-07-26

Time In:
Time Out:

Date of Onset: 09-01-25

Diagnosis:

M79.671 Pain in right foot
 M79.672 Pain in left foot
 M25.562 Pain in left knee
 M22.2X2 Patellofemoral disorders, left knee
 M54.50 Low back pain, unspecified

SUBJECTIVE:

Comments: 2/10 R foot pain currently

2/24: Patient states that her pain hovers around 1/10 and sometimes its also gone which feels very weird as she is so used to the pain. She is doing her HEP everyday.

2/19/26: Pt reports that her feet are feeling better. She has less issues at rest, but standing still bothers her. She does have a headache, but feels that that is not related.

2/17: Patient went to Zoo over the weekend and her feet hurt but she is doing the exercises. She feels good with exercises. R heel pain 2/10

Patient saw MD last week after talking to her therapists. X ray was taken which showed she has arthritis and disc herniation but PCP doesn't think patient needs new imaging as they don't anticipate big changes in it. Patient was given referral for her lumbar spine. Patient reports being "good for a while" initially but experienced a flare-up after increasing exercise activities over the summer, including swimming and yoga. Patient notes difficulty isolating pelvic floor muscles during breathing exercises, with tendency to activate upper abdominal muscles instead. Sleep quality is affected, requiring soft padding for comfort. Patient demonstrates understanding of exercise modifications and reports compliance with wall stretches when feeling stiff.

Contraindications:

- None

ACTIVITY LOG:

Time Based Activities	Comment	Status	Sets	Reps	Weight	Time	Done Today	Special	CPT
Myofascial Release	piriformis bil	Active				4	Yes		Neuro Re-Ed (97112) - [B02]

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Time Based Activities	Comment	Status	Sets	Reps	Weight	Time	Done Today	Special	CPT
hip flexor stretch	2x30"	Active				4	Yes		Neuro Re-Ed (97112) - [B02]
Joint Mobilization	PA lumbar glides/ SI joint AP and inferior glide R side	Active				5	Yes		Neuro Re-Ed (97112) - [B02]
Patient education		Active				6	Yes		Therapeutic Activity (97530) - [B12]
Taping	Heel support	Active				6	Yes		Neuro Re-Ed (97112) - [B02]
dead bugs		Active				5	Yes		Neuro Re-Ed (97112) - [B02]
Soft Tissue Mobilization	R heel and plantar fascia with gua sha release	Active				10	Yes		Neuro Re-Ed (97112) - [B02]

ASSESSMENT:

Patient consents to treatment plan and goals and gives verbal informed consent. 2/14: Plantarfascia and peroneal release with gua sha performed as patient c/o soreness and tightness. Taping for heel stabilization done today with care instructions given to patient. Dead bugs were added today and patient was appropriately challenged with them. She c/o snapping feeling in her R hip flexor area so hip flexor stretch was added as well.

2/19/26: Pt with improvements in her soft tissue irritability and core strength today.

2/17/26: Manual lumbar traction performed and patient tolerated it well. MFR performed on glutes/piriformis to release tightness.

2/12/26: Pt with improvements in her core strength and motor control.

2/5/26: Pt with reproduction of her pain with palpation of the Achilles tendon. With palpation she has signs and symptoms consistent with diastasis recti with pain reproduction with abdominal distention.

2/2/2026

The tx session began with cobra pose, followed by soft tissue mobilization the the thoracic and lumbar spine. Pt reported increased tenderness more distally along the lumbar spine. Childs pose and open books were performed to facilitate tissue extensibility of the thoracic and lumbar spine. Pt reported feeling decreased symptoms in bilateral feet after the tx session. Pt education provided on discussing low back findings with physician and whether or not imaging would be recommended. Pt's last MRI of the spine was performed 2 years ago.

1/29/26: Pt's back and feet symptoms continue to respond to McKenzie extension interventions. Her knee pain does improve with tibial IR mobilization, indicating mechanical left knee pain.

1/21/26: Today, the patient's foot pain, numbness, and weakness improves with lumbar PA glides as well as with myofascial release of the piriformis. These signs and symptoms are consistent with neurological origin of symptoms. At next visit, plan to continue to address lumbar radicular symptoms and work on improving core and hip strength.

Pt is a 42 year old female who reports gradual onset of B heel pain and L knee pain that has been ongoing since 9/2025. Pt demonstrates decreased B heel pain that is indicative of a relevant lumbar component as demonstrated by decreased B heel pain from a 6/10 to a 2/10 with pain moving from heel to just above calcaneus. Pt continues to report occasional numbness and tingling in feet with prolonged sitting.

Goals	Short-Long	Time Frame	Result	Comment
Patient will be independent in progressive monitored HEP.	Short Term	1 week	Met	
Improve position tolerance of walking on uneven terrain	Long Term	8 weeks	Ongoing	
Improve L single leg stance time to 30 seconds	Long Term	8 weeks	Ongoing	
Pt will stand 30 minutes pain free	Long Term	8 weeks	Partially Met	

Patient: Tatjana Hocke-Mirzashvili

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Goals	Short-Long	Time Frame	Result	Comment
			1-24%	
Patient will be independent with HEP	Short Term	4 weeks		
Patient will demonstrate increase in L/S AROM in all directions w/o pain	Long Term	8 weeks		
Centralization of radicular symptoms	Long Term	8 weeks		
Eliminate post innominate rotation on R Side	Short Term	4 weeks		

PLAN:

Tatjana will continue therapy as prescribed. This is visit # 24.

Best regards,

Namrah Quazi PT1360147

This document was electronically signed on 02-24-26 at 09:53a by Namrah Quazi PT1360147 Lic# 1360147



Endeavor Physical Therapy
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Daily Note

Date of Visit: 02-19-26
Patient Name: Hocke-Mirzashvili, Tatjana
Patient #: TX0458176

Therapist: Christobal Saenz
Referring MD: Martha Pyron
Phone: (512) 676-2500
Fax: (512) 406-7377

Date of Birth: 12-15-1982
Age: 43
Certification Period: 02-11-26 - 04-07-26

Time In:
Time Out:

Date of Onset: 09-01-25

Diagnosis:

M79.671 Pain in right foot
M79.672 Pain in left foot
M25.562 Pain in left knee
M22.2X2 Patellofemoral disorders, left knee
M54.50 Low back pain, unspecified

SUBJECTIVE:

Comments: 2/10 R foot pain currently

2/19/26: Pt reports that her feet are feeling better. She has less issues at rest, but standing still bothers her. She does have a headache, but feels that that is not related.

2/17: Patient went to Zoo over the weekend and her feet hurt but she is doing the exercises. She feels good with exercises. R heel pain 2/10

Patient saw MD last week after talking to her therapists. X ray was taken which showed she has arthritis and disc herniation but PCP doesn't think patient needs new imaging as they don't anticipate big changes in it. Patient was given referral for her lumbar spine. Patient reports being "good for a while" initially but experienced a flare-up after increasing exercise activities over the summer, including swimming and yoga. Patient notes difficulty isolating pelvic floor muscles during breathing exercises, with tendency to activate upper abdominal muscles instead. Sleep quality is affected, requiring soft padding for comfort. Patient demonstrates understanding of exercise modifications and reports compliance with wall stretches when feeling stiff.

Contraindications:

- None

ACTIVITY LOG:

Time Based Activities	Comment	Status	Sets	Reps	Weight	Time	Done Today	Special	CPT
Paloff Press						6	Yes		Therapeutic Activity (97530) - [B12]

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Time Based Activities	Comment	Status	Sets	Reps	Weight	Time	Done Today	Special	CPT
Bridge	3#	Active				4	Yes		Therapeutic Activity (97530) - [B12]
Braced Reverse Marching						6	Yes		Therapeutic Activity (97530) - [B12]
Repeated extension in lying	McKenzie extension level 4 (press up on hands) with belt	Active				4	Yes		Neuro Re-Ed (97112) - [B02]
Posterior Pelvic Tilt		Active				4	Yes		Neuro Re-Ed (97112) - [B02]
Joint Mobilization	PA lumbar glides/ SI joint AP and inferior glide R side	Active				12	Yes		Neuro Re-Ed (97112) - [B02]
Patient education		Active				4	Yes		Therapeutic Activity (97530) - [B12]

ASSESSMENT:

Patient consents to treatment plan and goals and gives verbal informed consent. 2/19/26: Pt with improvements in her soft tissue irritability and core strength today.

2/17/26: Manual lumbar traction performed and patient tolerated it well. MFR performed on glutes/piriformis to release tightness.

2/12/26: Pt with improvements in her core strength and motor control.

2/5/26: Pt with reproduction of her pain with palpation of the Achilles tendon. With palpation she has signs and symptoms consistent with diastasis recti with pain reproduction with abdominal distention.

2/2/2026

The tx session began with cobra pose, followed by soft tissue mobilization the the thoracic and lumbar spine. Pt reported increased tenderness more distally along the lumbar spine. Childs pose and open books were performed to facilitate tissue extensibility of the thoracic and lumbar spine. Pt reported feeling decreased symptoms in bilateral feet after the tx session. Pt education provided on discussing low back findings with physician and whether or not imaging would be recommended. Pt's last MRI of the spine was performed 2 years ago.

1/29/26: Pt's back and feet symptoms continue to respond to McKenzie extension interventions. Her knee pain does improve with tibial IR mobilization, indicating mechanical left knee pain.

1/21/26: Today, the patient's foot pain, numbness, and weakness improves with lumbar PA glides as well as with myofascial release of the piriformis. These signs and symptoms are consistent with neurological origin of symptoms. At next visit, plan to continue to address lumbar radicular symptoms and work on improving core and hip strength.

Pt is a 42 year old female who reports gradual onset of B heel pain and L knee pain that has been ongoing since 9/2025. Pt demonstrates decreased B heel pain that is indicative of a relevant lumbar component as demonstrated by decreased B heel pain from a 6/10 to a 2/10 with pain moving from heel to just above calcaneus. Pt continues to report occasional numbness and tingling in feet with prolonged sitting.

Goals	Short-Long	Time Frame	Result	Comment
Patient will be independent in progressive monitored HEP.	Short Term	1 week	Met	
Improve position tolerance of walking on uneven terrain	Long Term	8 weeks	Ongoing	
Improve L single leg stance time to 30 seconds	Long Term	8 weeks	Ongoing	
Pt will stand 30 minutes pain free	Long Term	8 weeks	Partially Met	

Patient: Tatjana Hocke-Mirzashvili

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Phone: (737) 843-7014 Fax: (737) 843-7016

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Goals	Short-Long	Time Frame	Result	Comment
			1-24%	
Patient will be independent with HEP	Short Term	4 weeks		
Patient will demonstrate increase in L/S AROM in all directions w/o pain	Long Term	8 weeks		
Centralization of radicular symptoms	Long Term	8 weeks		
Eliminate post innominate rotation on R Side	Short Term	4 weeks		

PLAN:

Tatjana will continue therapy as prescribed. This is visit # 23.

Best regards,

Christobal Saenz PT

This document was electronically signed on 02-19-26 at 10:35a by Christobal Saenz PT Lic# TX48758PT



Daily Note

Date of Visit: 02-17-26
Patient Name: Hocke-Mirzashvili, Tatjana
Patient #: TX0458176

Therapist: Namrah Quazi
Referring MD: Martha Pyron
 Phone: (512) 676-2500
 Fax: (512) 406-7377

Date of Birth: 12-15-1982
Age: 43
Certification Period: 02-11-26 - 04-07-26

Time In:
Time Out:

Date of Onset: 09-01-25

Diagnosis:

M79.671 Pain in right foot
 M79.672 Pain in left foot
 M25.562 Pain in left knee
 M22.2X2 Patellofemoral disorders, left knee
 M54.50 Low back pain, unspecified

SUBJECTIVE:

Comments: 2/10 R foot pain currently

2/17: Patient went to Zoo over the weekend and her feet hurt but she is doing the exercises. She feels good with exercises. R heel pain 2/10

Patient saw MD last week after talking to her therapists. X ray was taken which showed she has arthritis and disc herniation but PCP doesn't think patient needs new imaging as they don't anticipate big changes in it. Patient was given referral for her lumbar spine. Patient reports being "good for a while" initially but experienced a flare-up after increasing exercise activities over the summer, including swimming and yoga. Patient notes difficulty isolating pelvic floor muscles during breathing exercises, with tendency to activate upper abdominal muscles instead. Sleep quality is affected, requiring soft padding for comfort. Patient demonstrates understanding of exercise modifications and reports compliance with wall stretches when feeling stiff.

Contraindications:

- None

ACTIVITY LOG:

Time Based Activities	Comment	Status	Sets	Reps	Weight	Time	Done Today	Special	CPT
Lumbar and SI glides	R PA and inferior SI glides; PA glides L/S globally	Active				8	Yes		Neuro Re-Ed (97112) - [B02]
Myofascial	piriformis bil	Active				10	Yes		Neuro Re-Ed

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Time Based Activities	Comment	Status	Sets	Reps	Weight	Time	Done Today	Special	CPT
Release									(97112) - [B02]
Repeated extension in lying	McKenzie extension level 4 (press up on hands) with belt	Active				4	Yes		Neuro Re-Ed (97112) - [B02]
Posterior Pelvic Tilt		Active				4	Yes		Neuro Re-Ed (97112) - [B02]
Manual lumbar tracti		Active				6	Yes		Neuro Re-Ed (97112) - [B02]
Joint Mobilization	PA lumbar glides/ SI joint AP and inferior glide R side	Active				12	Yes		Neuro Re-Ed (97112) - [B02]

ASSESSMENT:

Patient consents to treatment plan and goals and gives verbal informed consent. 2/17/26: Manual lumbar traction performed and patient tolerated it well. MFR performed on glutes/piriformis to release tightness.

2/12/26: Pt with improvements in her core strength and motor control.

2/5/26: Pt with reproduction of her pain with palpation of the Achilles tendon. With palpation she has signs and symptoms consistent with diastasis recti with pain reproduction with abdominal distention.

2/2/2026

The tx session began with cobra pose, followed by soft tissue mobilization the the thoracic and lumbar spine. Pt reported increased tenderness more distally along the lumbar spine. Childs pose and open books were performed to facilitate tissue extensibility of the thoracic and lumbar spine. Pt reported feeling decreased symptoms in bilateral feet after the tx session. Pt education provided on discussing low back findings with physician and whether or not imaging would be recommended. Pt's last MRI of the spine was performed 2 years ago.

1/29/26: Pt's back and feet symptoms continue to respond to McKenzie extension interventions. Her knee pain does improve with tibial IR mobilization, indicating mechanical left knee pain.

1/21/26: Today, the patient's foot pain, numbness, and weakness improves with lumbar PA glides as well as with myofascial release of the piriformis. These signs and symptoms are consistent with neurological origin of symptoms. At next visit, plan to continue to address lumbar radicular symptoms and work on improving core and hip strength.

Pt is a 42 year old female who reports gradual onset of B heel pain and L knee pain that has been ongoing since 9/2025. Pt demonstrates decreased B heel pain that is indicative of a relevant lumbar component as demonstrated by decreased B heel pain from a 6/10 to a 2/10 with pain moving from heel to just above calcaneus. Pt continues to report occasional numbness and tingling in feet with prolonged sitting.

Goals	Short-Long	Time Frame	Result	Comment
Patient will be independent in progressive monitored HEP.	Short Term	1 week	Met	
Improve position tolerance of walking on uneven terrain	Long Term	8 weeks	Ongoing	
Improve L single leg stance time to 30 seconds	Long Term	8 weeks	Ongoing	
Pt will stand 30 minutes pain free	Long Term	8 weeks	Partially Met 1-24%	
Patient will be independent with HEP	Short Term	4 weeks		
Patient will demonstrate increase in L/S AROM in all directions w/o pain	Long Term	8 weeks		
Centralization of radicular symptoms	Long Term	8 weeks		
Eliminate post innominate rotation on R Side	Short Term	4 weeks		

Patient: Tatjana Hocke-Mirzashvili

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PLAN:

Tatjana will continue therapy as prescribed. This is visit # 22.

Best regards,

Namrah Quazi PT1360147

This document was electronically signed on 02-17-26 at 11:19a by Namrah Quazi PT1360147 Lic# 1360147



Endeavor Physical Therapy
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Daily Note

Date of Visit: 02-12-26
Patient Name: Hocke-Mirzashvili, Tatjana
Patient #: TX0458176

Therapist: Christobal Saenz
Referring MD: Martha Pyron
Phone: (512) 676-2500
Fax: (512) 406-7377

Date of Birth: 12-15-1982
Age: 43
Certification Period: 02-11-26 - 04-07-26

Time In:
Time Out:

Date of Onset: 09-01-25

Diagnosis:

M79.671 Pain in right foot
M79.672 Pain in left foot
M25.562 Pain in left knee
M22.2X2 Patellofemoral disorders, left knee
M54.50 Low back pain, unspecified

SUBJECTIVE:

Comments: 2/10 R foot pain currently

2/12/26: Pt reports that she is feeling overall good. She has a new prescription for her back. Her physician diagnosed her with a disc issue and arthritis. Her foot pain is around a 2/10 but it was about a 4/10 when she had to do a long drive.

Patient saw MD last week after talking to her therapists. X ray was taken which showed she has arthritis and disc herniation but PCP doesn't think patient needs new imaging as they don't anticipate big changes in it. Patient was given referral for her lumbar spine. Patient reports being "good for a while" initially but experienced a flare-up after increasing exercise activities over the summer, including swimming and yoga. Patient notes difficulty isolating pelvic floor muscles during breathing exercises, with tendency to activate upper abdominal muscles instead. Sleep quality is affected, requiring soft padding for comfort. Patient demonstrates understanding of exercise modifications and reports compliance with wall stretches when feeling stiff.

Contraindications:

- None

ACTIVITY LOG:

Time Based Activities	Comment	Status	Sets	Reps	Weight	Time	Done Today	Special	CPT
Isometric Crunch		Active				6	Yes		Therapeutic Activity (97530) - [B12]
Braced Reverse Marching						6	Yes		Therapeutic Activity (97530) -

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Time Based Activities	Comment	Status	Sets	Reps	Weight	Time	Done Today	Special	CPT
									[B12]
Repeated extension in lying	McKenzie extension level 4 (press up on hands) with belt	Active				8	Yes		Neuro Re-Ed (97112) - [B02]
Bridge		Active				4	Yes		Therapeutic Activity (97530) - [B12]
Posterior Pelvic Tilt		Active				4	Yes		Neuro Re-Ed (97112) - [B02]
Joint Mobilization	PA lumbar glides/ SI joint AP and inferior glide R side	Active				12	Yes		Neuro Re-Ed (97112) - [B02]

ASSESSMENT:

Patient consents to treatment plan and goals and gives verbal informed consent. 2/12/26: Pt with improvements in her core strength and motor control.

2/5/26: Pt with reproduction of her pain with palpation of the Achilles tendon. With palpation she has signs and symptoms consistent with diastasis recti with pain reproduction with abdominal distention.

2/2/2026

The tx session began with cobra pose, followed by soft tissue mobilization the the thoracic and lumbar spine. Pt reported increased tenderness more distally along the lumbar spine. Childs pose and open books were performed to facilitate tissue extensibility of the thoracic and lumbar spine. Pt reported feeling decreased symptoms in bilateral feet after the tx session. Pt education provided on discussing low back findings with physician and whether or not imaging would be recommended. Pt's last MRI of the spine was performed 2 years ago.

1/29/26: Pt's back and feet symptoms continue to respond to McKenzie extension interventions. Her knee pain does improve with tibial IR mobilization, indicating mechanical left knee pain.

1/21/26: Today, the patient's foot pain, numbness, and weakness improves with lumbar PA glides as well as with myofascial release of the piriformis. These signs and symptoms are consistent with neurological origin of symptoms. At next visit, plan to continue to address lumbar radicular symptoms and work on improving core and hip strength.

Pt is a 42 year old female who reports gradual onset of B heel pain and L knee pain that has been ongoing since 9/2025. Pt demonstrates decreased B heel pain that is indicative of a relevant lumbar component as demonstrated by decreased B heel pain from a 6/10 to a 2/10 with pain moving from heel to just above calcaneus. Pt continues to report occasional numbness and tingling in feet with prolonged sitting.

Goals	Short-Long	Time Frame	Result	Comment
Patient will be independent in progressive monitored HEP.	Short Term	1 week	Met	
Improve position tolerance of walking on uneven terrain	Long Term	8 weeks	Ongoing	
Improve L single leg stance time to 30 seconds	Long Term	8 weeks	Ongoing	
Pt will stand 30 minutes pain free	Long Term	8 weeks	Partially Met 1-24%	
Patient will be independent with HEP	Short Term	4 weeks		
Patient will demonstrate increase in L/S AROM in all directions w/o pain	Long Term	8 weeks		
Centralization of radicular symptoms	Long Term	8 weeks		
Eliminate post innominate rotation on R Side	Short Term	4 weeks		

Patient: Tatjana Hocke-Mirzashvili

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PLAN:

Tatjana will continue therapy as prescribed. This is visit # 21.

Best regards,

Christobal Saenz PT

This document was electronically signed on 02-12-26 at 12:46p by Christobal Saenz PT Lic# TX48758PT



Progress Note

Date of Visit: 02-10-26
Patient Name: Hocke-Mirzashvili, Tatjana
Patient #: TX0458176

Therapist: Namrah Quazi
Referring MD: Martha Pyron
 Phone: (512) 676-2500
 Fax: (512) 406-7377

Date of Birth: 12-15-1982
Age: 43
Certification Period: 02-11-26 - 04-07-26

Time In:
Time Out:

Date of Onset: 09-01-25

Diagnosis:

M79.671 Pain in right foot
 M79.672 Pain in left foot
 M25.562 Pain in left knee
 M22.2X2 Patellofemoral disorders, left knee
 M54.50 Low back pain, unspecified

SUBJECTIVE:

Presenting Problems:

The patient reports:

- Difficulty ascending/descending stairs, Moderate
- Difficulty walking, Minimal
- Difficulty squatting, Moderate
- Back stiffness, Moderate
- Difficulty isolating pelvic floor muscles, Moderate

Comments: 2/10 R foot pain currently

Functional Status	Prior	Current
Activities of daily living	100%	50%

Contraindications:

- None

Patient saw MD last week after talking to her therapists. X ray was taken which showed she has arthritis and disc herniation but PCP doesn't think patient needs new imaging as they don't anticipate big changes in it. Patient was given referral for her lumbar spine. Patient reports being "good for a while" initially but experienced a flare-up after increasing exercise activities over the summer, including swimming and yoga. Patient notes difficulty isolating pelvic floor muscles during breathing exercises, with tendency to activate upper abdominal muscles instead. Sleep quality is affected, requiring soft padding for comfort. Patient demonstrates understanding of exercise modifications and reports compliance with wall stretches when feeling stiff.

**OBJECTIVE:
LUMBAR EVALUATION**

RANGE OF MOTION:

2/10 PN:

- Spinal flexion: Patient reports whole body pain/tightness during forward bending with toes up
- Side bending: Right side bending causes discomfort, left side bending tolerated better
- Spinal rotation: Left rotation causes some discomfort in opposite side, right rotation well tolerated

MANUAL MUSCLE TEST:

Lower Extremity	Initial
MMT	02-10-26
Right Hip Abduction	3
Left Hip Abduction	3

PALPATION:

SI joint: R AGMR

R post innominate rotation

TTP: R SI joint.

TTP: Bil lumbar paraspinals

FUNCTION/OBSERVATION:

c/o pain and sensitivity in bil heels

SPECIAL TESTS:

- Straight leg raise test: Positive on left side with pain reported in left side/back area
 - Left leg straight leg raise with neck flexion: Increased pain reported in neck area
- Slump test: BIL negative (muscle stretch)

JOINT MOBILITY:

L/S PA glides: Hypomobile and painful lower L/S and L5-S1

ACTIVITY LOG:

Time Based Activities	Comment	Status	Sets	Reps	Weight	Time	Done Today	Special	CPT
Reassessment	Lumbar assessment	Active				15	Yes		Therapeutic Activity (97530) - [B12]
Shotgun SI glide	using dowel	Active				3	Yes		Neuro Re-Ed (97112) - [B02]
Myofascial Release	piriformis	Active				5	Yes		Neuro Re-Ed (97112) - [B02]
Joint Mobilization	PA lumbar glides/ SI joint AP and inferior glide R side	Active				6	Yes		Neuro Re-Ed (97112) - [B02]

Patient: Tatjana Hocke-Mirzashvili

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Time Based Activities	Comment	Status	Sets	Reps	Weight	Time	Done Today	Special	CPT
Patient Education	clinical findings/POC/prognosis/all patient questions were answered	Active				12	Yes		Therapeutic Activity (97530) - [B12]
Pelvic Floor Exercises		Active				2	Yes		Neuro Re-Ed (97112) - [B02]

ASSESSMENT:

Patient demonstrates varied progress across multiple conditions being treated concurrently. For pelvic floor dysfunction, patient shows good understanding of breathing techniques and muscle isolation exercises, though continues to struggle with proper muscle activation and compensatory patterns. Patient reports difficulty separating pelvic floor contractions from upper abdominal muscle activation, indicating need for continued neuromuscular re-education. Patient demonstrates good compliance with home exercise program and motivation for recovery. Patient would benefit from continued skilled therapy to address neuromuscular re-education needs, manual therapy for tissue mobility, and progressive strengthening while managing multiple concurrent conditions. The patient's rehab potential is good. The patient's discharge prognosis is good.

The patient appears motivated and is able to perform exercises correctly with verbal cues only. The patient's progress towards goals is good and her tolerance to treatment is good. Patient consents to treatment plan and goals and gives verbal informed consent.

Goals	Short-Long	Time Frame	Result	Comment
Patient will be independent in progressive monitored HEP.	Short Term	1 week	Met	
Improve position tolerance of walking on uneven terrain	Long Term	8 weeks	Ongoing	
Improve L single leg stance time to 30 seconds	Long Term	8 weeks	Ongoing	
Pt will stand 30 minutes pain free	Long Term	8 weeks	Partially Met 1-24%	
Patient will be independent with HEP	Short Term	4 weeks		
Patient will demonstrate increase in L/S AROM in all directions w/o pain	Long Term	8 weeks		
Centralization of radicular symptoms	Long Term	8 weeks		
Eliminate post innominate rotation on R Side	Short Term	4 weeks		

PLAN:

The patient's treatment will include joint and soft tissue mobilization, neuromuscular re-education, therapeutic activities, Dry Needling, therapeutic exercises, therapeutic activities, gait training and e-Stim unattended no wound. The patient will continue therapy 2 times per week for 8 weeks, for an additional 16 visits and a total of 36 visits since start of care. This is visit # 20.

Thank you for being a valued partner. We will keep you updated on this patient's progress.

Best regards,

Namrah Quazi PT1360147

Patient: Tatjana Hocke-Mirzashvili

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This document was electronically signed on 02-10-26 at 02:50p by Namrah Quazi PT1360147 Lic# 1360147

Patient: Tatjana Hocke-Mirzashvili
1120 Cottonwood Creek Trl Bldg B Ste 220 Cedar Park, TX 786137860
Phone: (737) 843-7014 Fax: (737) 843-7016
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PLAN OF CARE

Date of Visit: 02-10-26

Referral Source: Martha Pyron
Phone: (512) 676-2500
Fax: (512) 406-7377

Patient Name: Tatjana Hocke-Mirzashvili

Patient Number: TX0458176

Date of Birth: 12-15-1982

Date of Onset: 09-01-25

Certification Period: 02-11-26 - 04-07-26

Diagnosis:

M79.671 Pain in right foot
M79.672 Pain in left foot
M25.562 Pain in left knee
M22.2X2 Patellofemoral disorders, left knee
M54.50 Low back pain, unspecified

ASSESSMENT:

Patient demonstrates varied progress across multiple conditions being treated concurrently. For pelvic floor dysfunction, patient shows good understanding of breathing techniques and muscle isolation exercises, though continues to struggle with proper muscle activation and compensatory patterns. Patient reports difficulty separating pelvic floor contractions from upper abdominal muscle activation, indicating need for continued neuromuscular re-education. Patient demonstrates good compliance with home exercise program and motivation for recovery. Patient would benefit from continued skilled therapy to address neuromuscular re-education needs, manual therapy for tissue mobility, and progressive strengthening while managing multiple concurrent conditions. She is aware of her diagnosis/prognosis. The plans and goals have been developed and discussed with the patient.

REHAB POTENTIAL:

The patient's rehab potential is good.

PLAN:

The patient will continue therapy 2 times per week for 8 weeks, for an additional 16 visits and a total of 36 visits since start of care. This may include the following treatments:

- joint and soft tissue mobilization
- neuromuscular re-education
- therapeutic activities
- Dry Needling
- therapeutic exercises
- therapeutic activities
- gait training
- e-Stim unattended no wound

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Goals	Short-Long	Time Frame	Result	Comment
Patient will be independent in progressive monitored HEP.	Short Term	1 week	Met	
Improve position tolerance of walking on uneven terrain	Long Term	8 weeks	Ongoing	
Improve L single leg stance time to 30 seconds	Long Term	8 weeks	Ongoing	
Pt will stand 30 minutes pain free	Long Term	8 weeks	Partially Met 1-24%	
Patient will be independent with HEP	Short Term	4 weeks		
Patient will demonstrate increase in L/S AROM in all directions w/o pain	Long Term	8 weeks		
Centralization of radicular symptoms	Long Term	8 weeks		
Eliminate post innominate rotation on R Side	Short Term	4 weeks		

Namrah Quazi PT1360147

This document was electronically signed on 02-10-26 at 02:50p by Namrah Quazi PT1360147 Lic# 1360147

This is just for your information - signature not required.

Patient: Tatjana Hocke-Mirzashvili
1120 Cottonwood Creek Trl Bldg B Ste 220 Cedar Park, TX 786137860
Phone: (737) 843-7014 Fax: (737) 843-7016
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Daily Note

Date of Visit: 02-05-26
Patient Name: Hocke-Mirzashvili, Tatjana
Patient #: TX0458176

Therapist: Christobal Saenz
Referring MD: Martha Pyron
 Phone: (512) 676-2500
 Fax: (512) 406-7377

Date of Birth: 12-15-1982
Age: 43
Certification Period: 01-08-26 - 02-18-26

Time In:
Time Out:

Date of Onset: 09-01-25

Diagnosis:

M79.671 Pain in right foot
 M79.672 Pain in left foot
 M25.562 Pain in left knee
 M22.2X2 Patellofemoral disorders, left knee

SUBJECTIVE:

Comments: 2/10 R foot pain currently

2/5/26: Pt reports that she is doing a little bit better. Pt reports that traction helped after her last session. She now feels that there is two types of pain in her foot, one is tingling and the other is a pulling in her foot.

2/2/2026

Pt presents to clinic today reporting that she has been feeling about the same as her last visit. No change in symptoms.

1/29/26: Pt reports that patient reports that she is "feeling fine." Her pain is better. She was pain free until she started driving in her car after her last session. Her routine got thrown off due to the weather and she wasn't able to do her HEP. As a result she feels a bit more pain today.

1/21/26: Pt reports that her pain is around a 3/10 today. It is getting better and feels stable but still bothers her especially when she walks more. McKenzie extension has been improving her symptoms.

Pt is a 42 year old female who reports gradual onset of B heel pain and L knee pain that has been ongoing since 9/2025. Pt reports decreasing foot pain since performing HEP of repeated extension in lying more. Pt reports feeling 50% better overall with pain shifting from heel pad to back of heel and outside of feet. Pt reports a significant decrease in B heel pain since initiating her HEP more. Pt continues to decline dry needling directly to feet.

Contraindications:

- None

OBJECTIVE:

NT this visit.

ACTIVITY LOG:

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Time Based Activities	Comment	Status	Sets	Reps	Weight	Time	Done Today	Special	CPT
Repeated extension in lying	McKenzie extension level 4 (press up on hands) Centralizes symptoms, causes some low back tension that diminishes quickly	Active				6	Yes		Neuro Re-Ed (97112) - [B02]
Joint Mobilization	PA lumbar glides	Active				8	Yes		Neuro Re-Ed (97112) - [B02]
Patient Education	importance of core and glute/hamstring strengthening, as well as stretching and mobility exercises for the thoracic and lumbar spine, as well as hip flexors.	Active				8	Yes		Therapeutic Activity (97530) - [B12]
Heel Raise	X 30	Active				6	Yes		Therapeutic Activity (97530) - [B12]
Bridge		Active				4	Yes		Therapeutic Activity (97530) - [B12]
Pelvic Floor Exercises		Active				4	Yes		Neuro Re-Ed (97112) - [B02]
Posterior Pelvic Tilt		Active				4	Yes		Neuro Re-Ed (97112) - [B02]

ASSESSMENT:

Patient consents to treatment plan and goals and gives verbal informed consent. 2/5/26: Pt with reproduction of her pain with palpation of the Achilles tendon. With palpation she has signs and symptoms consistent with diastasis recti with pain reproduction with abdominal distention.

2/2/2026

The tx session began with cobra pose, followed by soft tissue mobilization the the thoracic and lumbar spine. Pt reported increased tenderness more distally along the lumbar spine. Childs pose and open books were performed to facilitate tissue extensibility of the thoracic and lumbar spine. Pt reported feeling decreased symptoms in bilateral feet after the tx session. Pt education provided on discussing low back findings with physician and whether or not imaging would be recommended. Pt's last MRI of the spine was performed 2 years ago.

1/29/26: Pt's back and feet symptoms continue to respond to McKenzie extension interventions. Her knee pain does improve with tibial IR mobilization, indicating mechanical left knee pain.

1/21/26: Today, the patient's foot pain, numbness, and weakness improves with lumbar PA glides as well as with myofascial release of the piriformis. These signs and symptoms are consistent with neurological origin of symptoms. At next visit, plan to continue to address lumbar radicular symptoms and work on improving core and hip strength.

Pt is a 42 year old female who reports gradual onset of B heel pain and L knee pain that has been ongoing since 9/2025. Pt demonstrates decreased B heel pain that is indicative of a relevant lumbar component as demonstrated by decreased B heel pain from a 6/10 to a 2/10 with pain moving from heel to just above calcaneus. Pt continues to report occasional numbness and tingling in feet with prolonged sitting.

Patient: Tatjana Hocke-Mirzashvili

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Phone: (737) 843-7014 Fax: (737) 843-7016

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Goals	Short-Long	Time Frame	Result	Comment
Patient will be independent in progressive monitored HEP.	Short Term	1 week	Met	
Improve position tolerance of walking on uneven terrain	Long Term	8 weeks	Ongoing	
Improve L single leg stance time to 30 seconds	Long Term	8 weeks	Ongoing	
Pt will stand 30 minutes pain free	Long Term	8 weeks	Partially Met 1-24%	

PLAN:

Tatjana will continue therapy as prescribed. This is visit # 19. Continue with current POC.

Best regards,

Christobal Saenz PT

This document was electronically signed on 02-05-26 at 09:54a by Christobal Saenz PT Lic# TX48758PT



Daily Note

Date of Visit: 02-02-26
Patient Name: Hocke-Mirzashvili, Tatjana
Patient #: TX0458176

Therapist: Daniel Ciezobka
Referring MD: Martha Pyron
 Phone: (512) 676-2500
 Fax: (512) 406-7377

Date of Birth: 12-15-1982
Age: 43
Certification Period: 01-08-26 - 02-18-26

Time In:
Time Out:

Date of Onset: 09-01-25

Diagnosis:

M79.671 Pain in right foot
 M79.672 Pain in left foot
 M25.562 Pain in left knee
 M22.2X2 Patellofemoral disorders, left knee

SUBJECTIVE:

Comments: 2/10 R foot pain currently

2/2/2026

Pt presents to clinic today reporting that she has been feeling about the same as her last visit. No change in symptoms.

1/29/26: Pt reports that patient reports that she is "feeling fine." Her pain is better. She was pain free until she started driving in her car after her last session. Her routine got thrown off due to the weather and she wasn't able to do her HEP. As a result she feels a bit more pain today.

1/21/26: Pt reports that her pain is around a 3/10 today. It is getting better and feels stable but still bothers her especially when she walks more. McKenzie extension has been improving her symptoms.

Pt is a 42 year old female who reports gradual onset of B heel pain and L knee pain that has been ongoing since 9/2025. Pt reports decreasing foot pain since performing HEP of repeated extension in lying more. Pt reports feeling 50% better overall with pain shifting from heel pad to back of heel and outside of feet. Pt reports a significant decrease in B heel pain since initiating her HEP more. Pt continues to decline dry needling directly to feet.

Contraindications:

- None

OBJECTIVE:

NT this visit.

ACTIVITY LOG:

Time Based Activities	Comment	Status	Sets	Reps	Weight	Time	Done Today	Special	CPT
Repeated extension in lying	McKenzie extension level 4	Active				6	Yes		Ther-Ex (97110) - [B01]

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Time Based Activities	Comment	Status	Sets	Reps	Weight	Time	Done Today	Special	CPT
	(press up on hands) Centralizes symptoms, causes some low back tension that diminishes quickly								
Child pose	1x15	Active				3	Yes		Ther-Ex (97110) - [B01]
Open Books	1x15 bilaterally	Active				4	Yes		Ther-Ex (97110) - [B01]
Joint Mobilization	PA lumbar glides Left knee tibial IR (not performed today)	Active				6	Yes		Neuro Re-Ed (97112) - [B02]
Soft Tissue Mobilization	TrP lumbar spine Manual lumbar traction	Active				15	Yes		Neuro Re-Ed (97112) - [B02]
Patient Education	importance of core and glute/hamstring strengthening, as well as stretching and mobility exercises for the thoracic and lumbar spine, as well as hip flexors.	Active				8	Yes		Therapeutic Activity (97530) - [B12]

ASSESSMENT:

Patient consents to treatment plan and goals and gives verbal informed consent. 2/2/2026

The tx session began with cobra pose, followed by soft tissue mobilization the the thoracic and lumbar spine. Pt reported increased tenderness more distally along the lumbar spine. Childs pose and open books were performed to facilitate tissue extensibility of the thoracic and lumbar spine. Pt reported feeling decreased symptoms in bilateral feet after the tx session. Pt education provided on discussing low back findings with physician and whether or not imaging would be recommended. Pt's last MRI of the spine was performed 2 years ago.

1/29/26: Pt's back and feet symptoms continue to respond to McKenzie extension interventions. Her knee pain does improve with tibial IR mobilization, indicating mechanical left knee pain.

1/21/26: Today, the patient's foot pain, numbness, and weakness improves with lumbar PA glides as well as with myofascial release of the piriformis. These signs and symptoms are consistent with neurological origin of symptoms. At next visit, plan to continue to address lumbar radicular symptoms and work on improving core and hip strength.

Pt is a 42 year old female who reports gradual onset of B heel pain and L knee pain that has been ongoing since 9/2025. Pt demonstrates decreased B heel pain that is indicative of a relevant lumbar component as demonstrated by decreased B heel pain from a 6/10 to a 2/10 with pain moving from heel to just above calcaneus. Pt continues to report occasional numbness and tingling in feet with prolonged sitting.

Goals	Short-Long	Time Frame	Result	Comment
Patient will be independent in progressive monitored HEP.	Short Term	1 week	Met	
Improve position tolerance of walking on uneven terrain	Long Term	8 weeks	Ongoing	

Patient: Tatjana Hocke-Mirzashvili

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Goals	Short-Long	Time Frame	Result	Comment
Improve L single leg stance time to 30 seconds	Long Term	8 weeks	Ongoing	
Pt will stand 30 minutes pain free	Long Term	8 weeks	Partially Met 1-24%	

PLAN:

Tatjana will continue therapy as prescribed. This is visit # 18. Continue with current POC.

Best regards,

Daniel Ciezobka PT

This document was electronically signed on 02-02-26 at 02:35p by Daniel Ciezobka PT Lic# 1407386

Diane Hodges Popps PT

This document was electronically cosigned on 02-02-26 at 05:36p by Diane Hodges Popps PT Lic# TX1218117PT



Daily Note

Date of Visit: 01-29-26
Patient Name: Hocke-Mirzashvili, Tatjana
Patient #: TX0458176

Therapist: Christobal Saenz
Referring MD: Martha Pyron
 Phone: (512) 676-2500
 Fax: (512) 406-7377

Date of Birth: 12-15-1982
Age: 43
Certification Period: 01-08-26 - 02-18-26

Time In:
Time Out:

Date of Onset: 09-01-25

Diagnosis:

M79.671 Pain in right foot
 M79.672 Pain in left foot
 M25.562 Pain in left knee
 M22.2X2 Patellofemoral disorders, left knee

SUBJECTIVE:

Comments: 2/10 R foot pain currently

1/29/26: Pt reports that patient reports that she is "feeling fine." Her pain is better. She was pain free until she started driving in her car after her last session. Her routine got thrown off due to the weather and she wasn't able to do her HEP. As a result she feels a bit more pain today.

1/21/26: Pt reports that her pain is around a 3/10 today. It is getting better and feels stable but still bothers her especially when she walks more. McKenzie extension has been improving her symptoms.

Pt is a 42 year old female who reports gradual onset of B heel pain and L knee pain that has been ongoing since 9/2025. Pt reports decreasing foot pain since performing HEP of repeated extension in lying more. Pt reports feeling 50% better overall with pain shifting from heel pad to back of heel and outside of feet. Pt reports a significant decrease in B heel pain since initiating her HEP more. Pt continues to decline dry needling directly to feet.

Contraindications:

- None

ACTIVITY LOG:

Time Based Activities	Comment	Status	Sets	Reps	Weight	Time	Done Today	Special	CPT
Patient Education	car ergonomics, posture	Active				8	Yes		Therapeutic Activity (97530) - [B12]
Repeated extension in lying	McKenzie extension level 4 (press up on hands)	Active				8	Yes		Therapeutic Activity (97530) - [B12]

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Time Based Activities	Comment	Status	Sets	Reps	Weight	Time	Done Today	Special	CPT
	Centralizes symptoms, causes some low back tension that diminishes quickly								
Joint Mobilization	PA lumbar glides Left knee tibial IR	Active				15	Yes		Neuro Re-Ed (97112) - [B02]
Lifting Technique		Active				8	Yes		Neuro Re-Ed (97112) - [B02]

ASSESSMENT:

Patient consents to treatment plan and goals and gives verbal informed consent. 1/29/26: Pt's back and feet symptoms continue to respond to McKenzie extension interventions. Her knee pain does improve with tibial IR mobilization, indicating mechanical left knee pain.

1/21/26: Today, the patient's foot pain, numbness, and weakness improves with lumbar PA glides as well as with myofascial release of the piriformis. These signs and symptoms are consistent with neurological origin of symptoms. At next visit, plan to continue to address lumbar radicular symptoms and work on improving core and hip strength.

Pt is a 42 year old female who reports gradual onset of B heel pain and L knee pain that has been ongoing since 9/2025. Pt demonstrates decreased B heel pain that is indicative of a relevant lumbar component as demonstrated by decreased B heel pain from a 6/10 to a 2/10 with pain moving from heel to just above calcaneus. Pt continues to report occasional numbness and tingling in feet with prolonged sitting.

Goals	Short-Long	Time Frame	Result	Comment
Patient will be independent in progressive monitored HEP.	Short Term	1 week	Met	
Improve position tolerance of walking on uneven terrain	Long Term	8 weeks	Ongoing	
Improve L single leg stance time to 30 seconds	Long Term	8 weeks	Ongoing	
Pt will stand 30 minutes pain free	Long Term	8 weeks	Partially Met 1-24%	

PLAN:

Tatjana will continue therapy as prescribed. This is visit # 17.

Best regards,

Christobal Saenz PT

This document was electronically signed on 01-29-26 at 10:03a by Christobal Saenz PT Lic# TX48758PT

Patient: Tatjana Hocke-Mirzashvili
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Daily Note

Date of Visit: 01-21-26
Patient Name: Hocke-Mirzashvili, Tatjana
Patient #: TX0458176

Therapist: Christobal Saenz
Referring MD: Martha Pyron
 Phone: (512) 676-2500
 Fax: (512) 406-7377

Date of Birth: 12-15-1982
Age: 43
Certification Period: 01-08-26 - 02-18-26

Time In:
Time Out:

Date of Onset: 09-01-25

Diagnosis:

M79.671 Pain in right foot
 M79.672 Pain in left foot
 M25.562 Pain in left knee
 M22.2X2 Patellofemoral disorders, left knee

SUBJECTIVE:

Comments: 2/10 R foot pain currently

1/21/26: Pt reports that her pain is around a 3/10 today. It is getting better and feels stable but still bothers her especially when she walks more. McKenzie extension has been improving her symptoms.

Pt is a 42 year old female who reports gradual onset of B heel pain and L knee pain that has been ongoing since 9/2025. Pt reports decreasing foot pain since performing HEP of repeated extension in lying more. Pt reports feeling 50% better overall with pain shifting from heel pad to back of heel and outside of feet. Pt reports a significant decrease in B heel pain since initiating her HEP more. Pt continues to decline dry needling directly to feet.

Contraindications:

- None

ACTIVITY LOG:

Time Based Activities	Comment	Status	Sets	Reps	Weight	Time	Done Today	Special	CPT
Patient Education		Active				8	Yes		Therapeutic Activity (97530) - [B12]
Repeated extension in lying	McKenzie extension level 4 (press up on hands) Centralizes symptoms, causes some low	Active				8	Yes		Therapeutic Activity (97530) - [B12]

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Time Based Activities	Comment	Status	Sets	Reps	Weight	Time	Done Today	Special	CPT
	back tension that diminishes quickly								
Joint Mobilization		Active				12	Yes		Neuro Re-Ed (97112) - [B02]
Myofascial Release	piriformis	Active				8	Yes		Neuro Re-Ed (97112) - [B02]

ASSESSMENT:

Patient consents to treatment plan and goals and gives verbal informed consent. 1/21/26: Today, the patient's foot pain, numbness, and weakness improves with lumbar PA glides as well as with myofascial release of the piriformis. These signs and symptoms are consistent with neurological origin of symptoms. At next visit, plan to continue to address lumbar radicular symptoms and work on improving core and hip strength.

Pt is a 42 year old female who reports gradual onset of B heel pain and L knee pain that has been ongoing since 9/2025. Pt demonstrates decreased B heel pain that is indicative of a relevant lumbar component as demonstrated by decreased B heel pain from a 6/10 to a 2/10 with pain moving from heel to just above calcaneus. Pt continues to report occasional numbness and tingling in feet with prolonged sitting.

Goals	Short-Long	Time Frame	Result	Comment
Patient will be independent in progressive monitored HEP.	Short Term	1 week	Met	
Improve position tolerance of walking on uneven terrain	Long Term	8 weeks	Ongoing	
Improve L single leg stance time to 30 seconds	Long Term	8 weeks	Ongoing	
Pt will stand 30 minutes pain free	Long Term	8 weeks	Partially Met 1-24%	

PLAN:

Tatjana will continue therapy as prescribed. This is visit # 16.

Best regards,

Christobal Saenz PT

This document was electronically signed on 01-21-26 at 11:15a by Christobal Saenz PT Lic# TX48758PT

Patient: Tatjana Hocke-Mirzashvili
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Daily Note

Date of Visit: 01-19-26
Patient Name: Hocke-Mirzashvili, Tatjana
Patient #: TX0458176

Therapist: Vicky Bledsoe
Referring MD: Martha Pyron
 Phone: (512) 676-2500
 Fax: (512) 406-7377

Date of Birth: 12-15-1982
Age: 43
Certification Period: 01-08-26 - 02-18-26

Time In:
Time Out:

Date of Onset: 09-01-25

Diagnosis:

M79.671 Pain in right foot
 M79.672 Pain in left foot
 M25.562 Pain in left knee
 M22.2X2 Patellofemoral disorders, left knee

SUBJECTIVE:

Comments: 2/10 R foot pain currently

Pt is a 42 year old female who reports gradual onset of B heel pain and L knee pain that has been ongoing since 9/2025. Pt reports decreasing foot pain since performing HEP of repeated extension in lying more. Pt reports feeling 50% better overall with pain shifting from heel pad to back of heel and outside of feet. Pt reports a significant decrease in B heel pain since initiating her HEP more. Pt continues to decline dry needling directly to feet.

Contraindications:

- None

ACTIVITY LOG:

Time Based Activities	Comment	Status	Sets	Reps	Weight	Time	Done Today	Special	CPT
treadmill	warm up backwards. speed to pt tolerance with 5% incline	Active				8	Yes		Therapeutic Activity (97530) - [B12]
Manual Therapy	IASTM to R distal lateral heel and fibularis mm	Active				15	Yes		Neuro Re-Ed (97112) - [B02]
Repeated extension in lying	3 X 10	Active				15	Yes		Therapeutic Activity (97530) - [B12]
Gastroc/Soleous Stretch	decline board	Active				4	Yes		Therapeutic Activity (97530) -

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Time Based Activities	Comment	Status	Sets	Reps	Weight	Time	Done Today	Special	CPT
									[B12]

ASSESSMENT:

Patient consents to treatment plan and goals and gives verbal informed consent. Pt is a 42 year old female who reports gradual onset of B heel pain and L knee pain that has been ongoing since 9/2025. Pt demonstrates decreased B heel pain that is indicative of a relevant lumbar component as demonstrated by decreased B heel pain from a 6/10 to a 2/10 with pain moving from heel to just above calcaneus. Pt continues to report occasional numbness and tingling in feet with prolonged sitting.

Goals	Short-Long	Time Frame	Result	Comment
Patient will be independent in progressive monitored HEP.	Short Term	1 week	Met	
Improve position tolerance of walking on uneven terrain	Long Term	8 weeks	Ongoing	
Improve L single leg stance time to 30 seconds	Long Term	8 weeks	Ongoing	
Pt will stand 30 minutes pain free	Long Term	8 weeks	Partially Met 1-24%	

PLAN:

Tatjana will continue therapy as prescribed. This is visit # 15.

Best regards,

Vicky Bledsoe PT

This document was electronically signed on 01-19-26 at 10:28a by Vicky Bledsoe PT Lic# 1098267



Daily Note

Date of Visit: 01-14-26
Patient Name: Hocke-Mirzashvili, Tatjana
Patient #: TX0458176

Therapist: Vicky Bledsoe
Referring MD: Martha Pyron
 Phone: (512) 676-2500
 Fax: (512) 406-7377

Date of Birth: 12-15-1982
Age: 43
Certification Period: 01-08-26 - 02-18-26

Time In:
Time Out:

Date of Onset: 09-01-25

Diagnosis:

M79.671 Pain in right foot
 M79.672 Pain in left foot
 M25.562 Pain in left knee
 M22.2X2 Patellofemoral disorders, left knee

SUBJECTIVE:

Comments: 6/10 R foot pain currently

Pt is a 42 year old female who reports gradual onset of B heel pain and L knee pain that has been ongoing since 9/2025. Pt reports increased R > L heel pain today after sitting in car taking a work meeting. Pt states she was resting on her heels with her ankle dorsiflexed. Pt states she was sittign in this position for 35 minutes.

Contraindications:

- None

ACTIVITY LOG:

Time Based Activities	Comment	Status	Sets	Reps	Weight	Time	Done Today	Special	CPT
Recumbent Bike	warm up	Active				8	Yes		Therapeutic Activity (97530) - [B12]
Manual Therapy	IASTM to R distal lateral heel and fibularis mm	Active				15	Yes		Neuro Re-Ed (97112) - [B02]
Repeated extension in lying	3 X 10	Active				10	Yes		Therapeutic Activity (97530) - [B12]
Gastroc/Soleous Stretch	decline board	Active				4	Yes		Therapeutic Activity (97530) - [B12]
Heel Raise	on decline board X 30	Active				4	Yes		Therapeutic Activity (97530) -

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Time Based Activities	Comment	Status	Sets	Reps	Weight	Time	Done Today	Special	CPT
									[B12]

ASSESSMENT:

Patient consents to treatment plan and goals and gives verbal informed consent. Pt is a 42 year old female who reports gradual onset of B heel pain and L knee pain that has been ongoing since 9/2025. Pt demonstrates decreased pain that is indicative of a relevant lumbar component as demonstrated by decreased B heel pain from a 6/10 to a 2/10 with pain moving from heel to just above calcaneus.

Goals	Short-Long	Time Frame	Result	Comment
Patient will be independent in progressive monitored HEP.	Short Term	1 week	Met	
Improve position tolerance of walking on uneven terrain	Long Term	8 weeks	Ongoing	
Improve L single leg stance time to 30 seconds	Long Term	8 weeks	Ongoing	
Pt will stand 30 minutes pain free	Long Term	8 weeks	Partially Met 1-24%	

PLAN:

Tatjana will continue therapy as prescribed. This is visit # 14.

Best regards,

Vicky Bledsoe PT

This document was electronically signed on 01-14-26 at 10:20a by Vicky Bledsoe PT Lic# 1098267

Patient: Tatjana Hocke-Mirzashvili

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